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07.10 Introduction

Service authorization is the process of issuing a written document, which enables enrolled service providers to initiate home and community-based services for participants. The service authorization is based on the finalized service plan for an individual CLTC participant.

The CLTC Participant's Rights and Responsibilities Statement includes several expectations that both a participant and CLTC must abide by if the participant receives waiver services. These expectations are critical to the effective delivery of services and should be carefully communicated to the participant.

Restrictions on amount of services that a waiver participant receives may be necessary. If this is determined to be the situation, such restrictions will be communicated to staff as interim policy and procedures.

07.20 Service Authorization Policy and Procedure

07.21 Waiver Services

The purpose of waiver services is to provide realistic options for participants who choose to remain at home and thus avoid unnecessary or premature nursing home placement.

Waiver services are provided by contracted and enrolled Medicaid providers to eligible participants and may include:

- Adult Day Health Care *
- Adult Day Health Care Nursing*
- Attendant Care
- Handheld Shower
- Case Management *
- Companion Services: Individual and Agency (* agency only)
- Environmental Modifications
- Enhanced Environmental Modifications
- Advanced Pest Control
- Home Delivered Meals *
- Nutritional Supplements
- *
- Personal Care Services *
- Personal Emergency Response System
- Pest Control
- Respite Care: Institutional *, Community Residential Care Facility *
- Telemonitoring

* indicates a scope of service is available

A scope of service exists for many of the waiver services and is part of the agreement which defines both provider and CLTC's responsibilities in the provision of waiver services. See the Chapter 7 Service Addendums for more information.

Self-Directed Waiver Services may not be provided by the following individuals:

1. A parent, stepparent, or foster parent of a minor Medicaid participant;
2. Any person who the legal responsibility of utilize their own assets for the care of the Medicaid Participant. (legally responsible adults)_

All other Waiver services may **not** be provided by the following individuals:

1. The spouse of the Medicaid participant (including married but separated).

Common law marriage is not recognized in South Carolina effective July 24, 2019; therefore, a significant other residing in a participant's household could be a paid caregiver if all other requirements are met.

2. A parent, stepparent, or foster parent of a minor Medicaid participant;
3. Any other legally responsible guardian (court ordered) of a Medicaid participant. Power of Attorney (POA) alone does not equate to being the legally responsible guardian;
4. A person who is the primary caregiver cannot be paid to provide respite. The reason for this is these two services are primarily designed to give relief to the primary caregiver.

Note: The Authorized Representative cannot serve as the supervisor of the paid caregiver and be the paid caregiver. This would mean the person was supervising himself/herself. Should there be any question as to whether a paid caregiver falls in any of the above listed categories, contact **the Area Administrator of the Area Office servicing the participant.**

Several factors influence authorization of waiver services. These factors include:

1. The participant's type of support system and the assistance provided.
2. The degree of assistance needed, and the time required to complete the care.
3. The participant's cognitive ability and its impact on the degree of assistance required and the time required to complete the care.
4. The participant's expressed need for assistance and the value the participant assigns to these needs.
5. The availability of other resources to meet these needs.
6. The most cost-effective way to meet these needs.

In summary, waiver services should only be authorized when there is a lack of available Non-Medicaid services to meet the participant's needs as specified in the CLTC Service Plan.

07.22 Choice of Waiver Service Provider(s)

A participant, and/or Authorized Representative must make a choice of provider for each waiver service. Note: If contacted by a non-Authorized Representative,

information can be obtained from this person, but information cannot be given or discussed with this person. Information regarding the participant can only be shared with the participant or Authorized Representative. Information discussed with a non-Authorized Representative is considered a HIPAA violation.

When the initial assessment is completed by in-home visit, the choice of case management provider is documented on the participant's Service Provider Report (Provider Choice Form) during the SCDHHS worker's assessment visit.

When the initial assessment is completed by phone, the choice of case management provider will be obtained by the **Case Manager II** when the case is transferred for enrollment.

The **Case Manager II** will document the provider choice(s) in order of preference in the narrative record. The provider choice form should be scanned into Phoenix. The remaining waiver services will be explained to the participant, Authorized Representative, and/or other permitted caregiver supports during the **Case Manager's** initial phone contact or visit at which time the participant, Authorized Representative, and/or other permitted caregiver supports may make a verbal choice of provider for a needed service or services.

- The verbal choice must be annotated on the Service Provider Report (Provider Choice Form) scanned into Phoenix and documented in the narrative record in order of preference.
- If a verbal choice cannot be obtained, a participant's Service Provider Report (Provider Choice Form) will be mailed to the participant and/or Authorized Representative. (Refer to Chapter 5, 05.20 Waiver Enrollment for more information.)

The participant Service Provider Report (Provider Choice Form) is discussed with the participant, Authorized Representative, and/or other permitted caregiver supports during the **Case Manager's Initial Visit**.

During this visit, the signature is secured for previous verbal choice(s).

- Subsequent changes or additions of providers are made verbally and documented in the narrative and need not be documented on the participant Service Provider Report (Provider Choice Form).
- The participant, Authorized Representative, and/or other permitted caregiver supports are to make all decisions regarding provider choice independently.
- The **Case Manager** must not influence a participant's choice of providers.
- If a participant is not physically or mentally able to complete and/or sign the participant Service Provider Report (Provider Choice Form), the Authorized Representative or other permitted caregiver supports may sign for the

- participant.
- The lack of a signature on the form will not preclude the participant from receiving waiver services.

Any time the participant, Authorized Representative, and/or other permitted caregiver support request an additional waiver service or a change in providers, the **Case Manager** will inform the participant, Authorized Representative, and/or other permitted caregiver supports of all available providers of the service(s). The **Case Manager** must narrate this information exchange as well as the choice of provider(s), in order of preference in the narrative record. The participant, Authorized Representative, and/or other permitted caregiver supports may request a change of provider for any service, including Case Management, at any time.

When creating a referral, the **Case Manager** will enter the provider choices in order of preference. Phoenix will automatically refer and notify the next Provider of Choice if a referral is declined, or an affirmative response has not been received from the first chosen provider within forty-eight (48) business hours.

Note: When a participant is subject to a waiting period for Medicaid Eligibility, the Case Manager will add a comment to the referral stating, “Participant is subject to a waiting period for Medicaid Eligibility and payment may be delayed.”

It is the responsibility of Central Office to keep a continuously updated Resource Directory for each waiver service available in that region.

PROCESS:

- SCDHHS worker will provide CLTC Brochure and CLTC Choosing a Case Manager brochure and secure participant/Authorized Representative signature on the initial Case Management Provider Choice Form if a home visit is completed.
- If the initial assessment is completed by phone, the **Case Manager II** will obtain the Case Management Provider Choice verbally during the enrollment process. These choices must be documented on the Service Provider Report (Provider Choice Form) and in the narrative record in order of preference. The provider choice form should also be scanned into Phoenix. The **Case Manager** will provide the CLTC Brochure and the CLTC Choosing a Case Manager Brochure at the Initial Visit.
- Upon enrollment, the **Case Manager II** will contact or visit participant/Authorized Representative for service planning.
- If service provider choices cannot be made by phone, **Case Manager II** will send appropriate Service Provider Report (Provider Choice Form) for the

negotiated services. **Case Manager II** will contact the participant and/or Authorized Representative **within seven (7) business days** from mailing the list regarding choice of providers. A copy of the signed provider choice form with the choice date should be scanned in Phoenix Scans under the Provider Scan Tag. **Upon Initial Visit** by the **Case Manager**, this form will be signed and dated by participant/Authorized Representative. If contact to complete service planning is by visit, the **Case Manager II** will have the choice form signed when choice is made.

- New cases may be transferred to the on-going **Case Manager** at any time during the month; however, the Initial Visit time frame requirements must be considered. Adequate time must be allowed for the **Case Manager** to complete the Initial Visit within the thirty (30) day timeliness requirement.
- ***Exception:** If the Initial Visit is due within five (5) business days of the Case Management referral, the **Case Manager** must complete the visit within ten (10) days of accepting the referral. The late visit activity will not be subject to sanction for timeliness if the Initial Visit is made within ten (10) business days of the referral acceptance.
- **Assigned Case Manager** will follow up on service provider choices and referrals/authorizations in process by the **State Case Manager**.

07.23 Authorization of Waiver Services

Before any waiver services are authorized, it is crucial that the **Case Manager** be familiar with all scopes of services and standards of services. Waiver services are based on a participant's needs as documented in the Service Plan. Services must receive prior approval from a designated State Worker. Waiver services for waiver participants of the Medicare-Medicaid Managed Plans (MMP) program are prior approved by the MMP designee.

Upon receipt of a participant's choice of provider (Section 07.22) and prior approval of a service (Section 07.27), the **Case Manager** must initiate referrals to the chosen providers **within two (2) business days**.

The referral process is completed in Phoenix and must include any special needs or preferences of the participant. A chosen provider must be given the opportunity to accept or reject the referral. Providers must respond **within 48 business hours**. Once a start date has been negotiated with a provider, the case manager has **two (2) business days** to complete and send the service provision form to the provider. (Exceptions to this policy is Adult Day Health Care which is referred but not authorized until receipt of the physician's order.) All waiver services are authorized by completion of the service provision form.

It is acceptable to authorize waiver services on the day of admission to or discharge from an institution (i.e., hospital, nursing home, rehabilitation center, psychiatric facility) The narrative must clearly document the need for such services.

Weekly services must have a Sunday start date for a change in service and not for the initial authorization for service.

The authorization period end date may or may not be indicated on the Service Provision Form. An end date is included when the service is required for a known duration, if less than thirty (30) days from the start date. The authorized end date is a valid date of service delivery. Authorizations without an end date are valid until a new service authorization or termination is issued to the provider.

PROCESS:

The assigned Case Manager:

- Contacts participant to discuss specific needs, provider choice and schedule of services.
- Updates appropriate sections of Phoenix to indicate need for services.
- Updates service plan as needed.
- Documents provider choices in the narrative in the order of preference.
- Requests prior approval in the service approval section of the service plan located under the Complete tab of the service plan.
- Once reviewed, **Case Manager** receives a notification on his/her dashboard. When this notification is received, the **Case Manager** will review the prior approval section (complete tab of the service plan) to determine subsequent actions.
- Begins referral process in Phoenix **within two (2) business days**, listing all provider choices in order of participant's preference.
- When notification of referral acceptance is received, negotiates start of services with the provider and participant/ Authorized Representative and completes service provision form **within two (2) business days**. It is crucial that the provision form be specific and accurate.
- Once the authorization is completed, the provider will have immediate access to the authorization in Phoenix.

- Service plans are available to providers in the Phoenix Provider Portal on a real time basis. Exceptions to this are attendant care and respite care. These processes are noted in the corresponding addendum.
- Verifies implementation of the service with participant or Authorized Representative no later than the next scheduled contact. Environmental Modifications and Handheld Shower is verified at the next contact following the authorization end date. Verification of services must be documented in the narrative.
- The case manager must **CONSULT SERVICE ADDENDUMS TO ENSURE COMPLIANCE WITH SPECIFIC SERVICE REQUIREMENTS.**

07.24 Verifying Service Delivery of Authorized Services

The Case Manager must address service delivery with the participant, and/or Authorized Representative during each scheduled contact. Care Call claims must be reviewed, and activities and observations reports run and reviewed. Follow-up with the provider(s) may be necessary to resolve questions or concerns related to service delivery. Problematic patterns noted in Care Call claims must be addressed. Refer to the P&P Chapter 5, Section 05.35, for more information regarding monitoring service delivery.

07.25 Change/Termination of Waiver Service

A change in service request should be initiated by the participant and/or Authorized Representative, not the service provider. If a service request/change comes from a provider, a telephone contact must be made to the participant or Authorized Representative for confirmation of need/choice. This must be documented in Phoenix narratives.

When an existing service authorization requires a change, other than a change of provider, the **Case Manager** completes a service termination and creates a new service authorization. An increase in service levels requires approval by designated **State Worker** or **MMP designee** for MMP waiver participants.

When a provider of services changes, the **Case Manager** must terminate the Service Provision Form with the previous provider and initiate a new service authorization for the chosen provider. The terminated Service Provision Form and the new Service Provision Form can be accessed in the Phoenix provider portal with the exception of Attendants, Individual Companion, and respite services.

When a service is changed for a weekly service, but the provider does not change, the authorization start date must be Sunday.

Services must be terminated if a participant no longer requires the service or if the participant becomes ineligible to receive the service. Waiver services may be terminated at any time during a re-evaluation cycle. Termination of a service will necessitate a revision of the service plan. Services will remain in effect through the date of termination on the Service Provision Form.

When services are involuntarily suspended, reduced, or terminated, the participant must be given a written appeal notice, as well as a written **ten (10) business day notice prior to the service being terminated**. The participant and/or Authorized Representative may file an appeal within thirty (30) calendar days of the CLTC Notification. The participant may be eligible to receive continued benefits/services pending an appeal hearing decision. The participant/ Authorized Representative must request an appeal and notify the assigned Case Manager or the CLTC Representative that he/ she wants services to continue prior to the date of action on the Notification form. Services may also be requested to be reinstated. The appeal request and reinstatement of benefits request must be received within ten (10) calendar days of the date of action indicated on the Notification form. Services will not be resumed while a participant's case is pending an appeal hearing if the date of action has passed. The participant and/or Authorized Representative will be informed that the participant may be responsible for the cost of services received during this period, if the appeal decision is not in the participant's favor.

The Notification Form is sent when services are being terminated, reduced, suspended, or denied. The notice must cite the specific federal regulation which supports CLTC's decision. These regulations will auto-populate on the Notification Form when the box is checked on the form to indicate these actions.

When a participant is determined to be financially ineligible, waiver services must be terminated on the last date of eligibility. This action must be documented in the narrative. A CLTC Notification Form will be sent to the participant/Authorized Representative; however, the Appeals Notice will not be included. Both the participant and provider must be notified verbally at the time of service termination if the termination is immediate. This verbal notification must be followed with a service termination to the provider and a notification form to the participant and appropriate parties. The Conversation feature in Phoenix may be used in instances where immediate information is not required and when appropriate.

When a medically ineligible decision is final for a waiver participant, waiver services must be terminated in accordance with proper notice to the provider.

When services are voluntarily suspended, reduced, or terminated, an appeals notice is not to be sent to the participant/ Authorized Representative. A CLTC Notification Form must be sent stating the change was made at the request of the participant and/or Authorized Representative.

If a requested service is denied, a CLTC Notification Form must be sent stating what service was denied and the appeals notice must be given. A ten (10) day notice is **not** required.

Notification Forms must be mailed within two (2) business days.

07.25.10 Temporary increase in services

The case manager may do an authorization for up to 10 units above the current approved level for an authorization that is not longer than one-week (seven (7) business days). The authorization must have a start and end date. For example, if the approved level for personal care services is 10 hours per week, the case manager can authorize 20 hours of personal care services with an end date no more than seven business days after the start date. In instances where the participant receives multiple services (personal care services, companion), the total for all combined services can only be increased up to 10 units. For example, if a participant receives, 10 hours per week personal care services, and 5 hours per week companion services, the Case Manager may increase the personal care services up to 20 hours (increase of 10 units) with an end date no more than seven business days after the start date while keeping all other service levels the same.

Note: If an increase is needed for longer than one week for units greater than the current approved level, a new service level must be approved.

07.26 Rationale for Increased Service Units

For Adult Day Health Care, Personal Care Services, Agency Companion, Individual Companion and/or Attendant Care:

When a participant is determined to need a change in the level of services provided, the **Case Manager** must review all services and strive to meet the needs of the participant. The need for the increase in service units must be justified by the assessment, narrative and service plan.

If the participant is not authorized for Adult Day Health Care OR the participant is authorized for Adult Day Health Care two (2) days or less per week, the services of Personal Care Services, Agency Companion, Individual Companion and/or Attendant Care can be approved by the State Reviewers in any combination not to exceed twenty (20) hours per week. The **Area Administrator or Lead Team** may approve any service combination up to 40 hours per week. Service combinations over 40 hours require Central Office approval.

If the participant is authorized for Adult Day Health Care three (3) or more days per

week, the services of the Personal Care Services, Agency Companion, Individual Companion and/or Attendant Care can be approved by the State Reviewers in any combination not to exceed fifteen (15) hours per week. The **Area Administrator or Lead Team** may approve any service combination up to 35 hours per week. Service combinations over 35 hours require Central Office approval.

The **Case Manager** must provide justification of the need for increased service units in the comments section of the approval request section of the Service Plan (located under the Complete tab) and submit the request through Phoenix to the **Area Administrator or Lead Team** for approval. The **Area Administrator or Lead Team** reviewing the request will enter a comment indicating approval or denial. If the request exceeds the number of hours which may be approved by the **Area Administrator or Lead Team** and the need is justified, the request is sent to the **Central Office Waiver Service Exceptions** for review. The **Case Manager** will send an email request to WES@scdhhs.gov asking for review of the request after approval from the **Area Administrator or Lead Team** is obtained. The **Central Office Waiver Service Exception Team** will review the request in Phoenix, complete the review column and indicate the decision in the comment section. The **Case Manager** will be notified by email from the Central Office Waiver Service Exceptions once the request has been reviewed.

When **Central Office** Service Exception is needed for any service, the **Area Office** reviewers will review the request and enter a comment with the recommendation regarding the request. The reviewer will leave the review column blank. The **Case Manager** must email the **Central Office Waiver Service Exceptions** at WES@scdhhs.gov to request review of the exception following the **Area Office** review. The **Central Office Designee** will either approve or deny the exception request and will notify the **Case Manager** by email.

Note: The prior approval review for Medicare-Medicaid Managed Plans (MMP)/waiver participants will be completed by the MMP Care Coordinator. The Case Manager will use the Phoenix Conversation feature to communicate the need for review by the MMP Care Coordinator. MMP Care Coordinators may approve service levels of Personal Care Services/ Attendant/ Companion/ Nutritional Supplements, etc. above the levels the CLTC Area Office may approve. A separate Central Office review is not needed in these cases.

07.27 Justifications for Prior Approval Requests

This policy describes the participant-specific information and justification required to be completed by the assigned case manager for the approval of a waiver service. Also included is the justification for Incontinence Supplies, a nonwaiver service.

The State approver will not require any additional information in the service justification section of the Service Plan, as this may inhibit person-centered

planning and/or delay service provision to the participant.

In addition to the justification, each prior approval service request requires a narrative documenting that the participant/Authorized Representative made the request. There should also be a narrative entry stating the outcome of the request and confirming that the referral process was initiated. Sections of the assessment – especially Home Assessment/Home Needs – as well as Service Plan Interventions should be updated to reflect service needs **BEFORE** the prior approval request is submitted.

NOTE: Diagnoses should not be listed in the justification unless related to a specific task for which the service is needed. Also, all service exceptions should be sent to the Area Administrator – or to the Lead Team in the Area Administrator’s absence -- for review prior to submitting to the Central Office Waiver Service Exceptions email group.

For service specific justification information refer to the service addendums at the end of this chapter.

07.27.10 Timeliness Standards of Justifications for Prior Approval Requests

To ensure timely provision of services to participants, the following timeliness standards apply:

- When a case manager submits a justification request and no additional information is required, the state approver has **three (3) business days** from the date of the request to respond with an approval or denial.
- The case manager has **three (3) business days** to respond to request for additional information.
- If no response is received from the case manager within **three (3) business days**, the state approver can deny the request.
- Failure of the case manager to respond timely to a service level request (without a valid reason) is sanctionable and a complaint should be submitted in Phoenix.

07.27.20 Pend or Deny a Justification

The following steps should be followed if the submitted justification does not

include all information required for a service (i.e., narratives, failure to update service plan or home assessment, etc.):

- The state approver should pend the request and notify the case manager of the missing information by email. An email must be sent to the case manager by his or her preferred email address as listed in Phoenix. Emails to the case manager must be copied to the case management provider and/or the case manager's supervisor.
- To pend the request: State approver flags the request by entering initials in the Worker Flag column. While in pending status, the state approver should not enter units (including zero) in the review column. Entering a "0" results in a denied service.
- If justification is complete and correct but the narrative and/or Service Plan is not, the state approver will pend the request until the case manager completes the narrative and/or updates the Service Plan.
- If a state approver is denying an increase in services, the state approver should enter the current amount authorized in the review column.
- Failure of the case manager to respond timely to a service level request (without a valid reason) is sanctionable and a complaint should be submitted in Phoenix.

07.30 Waiver Interactions with Other Programs

When a waiver participant is enrolled in or receives services from other programs, it creates special circumstances that dictate which services may be authorized. It is important to review these interactions to ensure appropriate and allowed services are being provided.

Note: The Hospice Benefit Interactions service limitations and the CRCF service restrictions do not apply for MMP/ waiver participants.

07.31 Waiver Interactions with Community Residential Care Facility

07.31.10 Waiver Participants Residing in a Community Residential Care Facility (CRCF)

A waiver participant may continue waiver services while residing in a CRCF if that specific facility meets waiver requirements. Prior to authorizing waiver services for the participant residing in a CRCF, the following questions must be asked of the CRCF Administrator or designee. This information must be documented in the narrative. If any answer is no, first determine why the answer is no. It may require working with the CRCF to see what can be accommodated, or reviewing the participant's documentation to determine that a "no" answer is appropriate. The preference is to keep a person in the waiver and explore other alternatives, such as looking for a different residential setting for the participant.

Note: A Boarding Home considered a community location. Since the Boarding Home is not licensed it is considered a normal rental residence. The following questions do not apply for boarding home residents.

All residential settings must have the following qualities:

- a) Legally enforceable agreement with same responsibilities and protections from eviction that tenants have under landlord/tenant law.
- b) Privacy in sleeping/living unit- Is there a curtain or other separation between sleeping units?
- c) Room entrance doors lockable by participant with only appropriate staff having keys.
- d) Choice of roommate or option to change rooms if roommate not compatible.
- e) Freedom to furnish and decorate their sleeping/living units.
- f) Freedom and support to control their own schedules/activities.

- g) Freedom to have access to food at any time? Substitute meal available upon return if out of the facility at meal time?
- h) Able to have visitors, of their choosing, at any time.
- i) Physically accessible- handicapped accessible?

07.31.20 **Relocation/ Termination of a Participant in a Non-Compliant Community Residential Care Facility**

Should relocation of a waiver participant be required in cases where the CRCF is not in compliance with waiver requirements, the **Case Manager** or **Nurse Consultant** will discuss all options with the participant, Authorized Representative and CRCF.

The **Case Manager** or **Nurse Consultant** will notify the **Area Administrator** and/or **Lead Team** if it is determined that a CLTC waiver participant is residing in a non-compliant CRCF. The **Area Administrator** or **Lead Team** will notify **Central Office Designee**.

The **Case Manager** or **Nurse Consultant** will:

- Provide a list to the participant of compliant CRCFs from the Nursing Facility Bed Locator.
- Explain alternative options for care including other non-waiver community resources or aging services.
- Explain the need to re-locate to a compliant facility in order to remain enrolled in waiver.
- Document all decisions. Send a CLTC Notification to participant and/or Authorized Representative providing a thirty (30) day notice indicating the need to move to a compliant setting.

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Refer to Waiver Interactions with Community Residential Care Facility (Refer to Service Addendum 7S.)

07.32 **Waiver Interactions with Medicaid State Plan Incontinence Supplies**

Note: Please refer to the State Plan Medicaid Incontinence Supplies Policy and Procedure Manual located in Phoenix HELP for complete information regarding case management of non-waiver Incontinence Supplies. Refer to the Medicare-Medicaid Managed Plans (MMP) Incontinence Supplies guidance for complete information regarding participants enrolled in MMP.

Supplies interactions.

When a recipient of State Plan Incontinence Supplies, is enrolled in a CLTC waiver program, the Incontinence Supplies application will be transferred from an Incontinence Supplies Vendor to the assigned **Waiver Case Manager or Area Office State Staff worker**. The **Case Manager or Area Office State Staff worker** will ensure the Incontinence Supplies monitoring authorization has been terminated.

If there is an open waiver case assigned to a **Case Manager or Area Office State Staff worker** and the participant needs incontinence supplies, the **Case Manager or Area Office State Staff worker** will create an Incontinence Supplies Application in Phoenix under the application tab. **An e-Referral or Centralized Intake referral should not be made.**

Creating an IS Application for a current waiver participant:

- Create an Incontinence Supplies application under “Applications” in Phoenix. The worker will indicate the participant is Medicaid eligible.
- Assign the application to the same CLTC worker assigned to the waiver application.
- The **Assigned Case Manager or Area Office State Staff worker** will complete the Incontinence Supplies assessment. The **Case Manager or Area Office State Staff worker** should indicate the assessment is a “phone assessment” by clicking the phone assessment box in Phoenix. The bowel/bladder section of the Community Choices waiver assessment can be used to complete the Incontinence Supplies assessment.
- **Do not complete the separate Incontinence Supplies Service Plan.**
- Add Incontinence Supplies to the Waiver Service Plan.
- Request prior approval via the service request section of the Service Plan in Phoenix.

Note: For CC waiver applicants, the Area Office State Staff worker assessing the case will proceed with completing the Incontinence Supplies referral and assessment, and then transfer the case to the Case Manager II/Lead Team Case Manager for enrollment.

Service Justification: State frequency of bowel or bladder incontinence and include “Assessment updated” if applicable. If participant has ostomy or catheter or both, include need due to leakage.

- Create the Incontinence Supplies referral. The worker will include a comment

on the Incontinence Supplies referral notifying the IS provider when there is a 30 day wait.

- The Incontinence Supplies provider will obtain the Physician's Certification of Incontinence after receiving the referral.
- The **Case Manager or Area Office State Staff worker** will create the Service Provision Form (authorization) after receiving notification of acknowledgment from the provider that the Physician's Certification of Incontinence has been obtained, which can be found under Forms in Phoenix.
- The Physician Certification of Incontinence Statement must be obtained by the IS provider prior to the **Case Manager or Area Office State Staff worker** creating and sending the Initial Service Provision Form (authorization) for Incontinence Supplies. It is the responsibility of the **Case Manager or Area Office State Staff worker** to make sure the provider acknowledgement in Phoenix states the initial Physician Certification of Incontinence Statement has been obtained by the provider prior to authorizing IS. The **assigned Case Manager or Area Office State Staff worker** will receive a Phoenix notification when the provider indicates the certification has been received.

The **Case Manager or Area Office State Staff worker** may view the acknowledgement in Phoenix Forms.

The initial Physician Certification of Incontinence Statement is valid up to 12 months for waiver participants. The provider is responsible for obtaining an updated Physician Certification of Incontinence Statement and notifying the **Case Manager or Area Office State Staff worker** through Phoenix when the updated form is received. It is the Incontinence Supplies provider responsibility to request termination of Incontinence Supplies if the Physician Certification of Incontinence form expires and is not renewed.

- A waiver participant may receive Incontinence Supplies in a Community Residential Care Facility.
- When Incontinence Supplies are being terminated the **Case Manager or Area Office State Staff worker** will send a CLTC Notification Form with a ten (10) business day notice to the participant/Authorized Representative prior to termination of IS. The notice must indicate the reason the supplies are being terminated.

When a waiver participant, who receives Incontinence Supplies, becomes a MMP member, the **Case Manager** will:

- Manage Incontinence Supplies application during the six (6) month continuity of care period. The six (6) month continuity of care period begins when the

participant becomes a MMP member.

The **MMP Care Coordinator** will:

- Contact the waiver **Case Manager** thirty (30) days prior to the end of the six (6) month period to inform the **Case Manager** when the **MMP Care Coordinator** will begin managing Incontinence Supplies for the participant. This discussion will be documented in the narrative by the **Case Manager**.
- Contact the waiver **Case Manager** when it is time for the Incontinence Supplies application to be transferred to the **MMP Care Coordinator**.

The waiver **Case Manager** will:

- Send a CLTC Notification to the participant explaining the Incontinence Supplies application will now be managed by the **MMP Care Coordinator**. The notification will include the **MMP Care Coordinator's** contact information.
- Terminate the Incontinence Supplies authorizations.
- Request the CLTC **Area Office Support Staff** re-assign the Incontinence Supplies application to the **MMP Care Coordinator**.
- Document all actions in the narrative.

The **MMP Care Coordinator** will terminate the Incontinence Supplies application in Phoenix.

When waiver applications are terminated but the participant remains Medicaid Eligible and continues to need Incontinence Supplies:

- The Incontinence Supplies Application and Incontinence Supplies Service Provision Forms will remain open in Phoenix.
- The Waiver **Case Manager or Area Office State Staff worker** will ensure an Initial Incontinence Supplies Assessment has been completed in Phoenix. If not present, the assessment must be completed prior to re-assigning the Incontinence Supplies Application.
- Any hard copy documents must be scanned into Phoenix Scans.
- The incontinence Supplies Application will be re-assigned to an Incontinence Supplies vendor for service coordination. The Case Manager will notify the Central Office Incontinence Supplies Specialist of the need to re-assign the application to the vendor via email.

Note: If the participant is transferring to a DDSN waiver, the **Case Manager or Area Office State Staff worker** will terminate the IS Application and authorizations. DDSN will then manage the Incontinence Supplies.

07.33 Waiver Interactions with Hospice

Waiver participants who are enrolled in Hospice Care may receive **limited** waiver services only. Participants receiving palliative care and are not enrolled in Hospice are eligible for all waiver services.

Service limitations apply regardless of whether Hospice is being providing under Medicare or Medicaid payment. Please refer to the waiver service matrix under Service Addendum 7T.

SERVICE ADDENDUMS

7A Adult Day Health Care

One (1) unit of service is a participant-day of Adult Day Health Care (ADHC) service, consisting of a minimum of five (5) hours in the care of the Adult Day Health Care Center.

Exceptions: The following are examples of cases where CLTC **may** consider a request for reimbursement even though the participant was not under the care of the center for the entire five (5) hours. In all such cases, the provider must advise the **Case Manager**. The **Case Manager** must notify the CLTC **Area Office Administrator/Lead Team Case Manager**. The contact must be documented in Phoenix.

- If the participant has a medical emergency, non-scheduled medical/doctor's visit, or need for urgent care.
- Inclement weather affecting the safe transport of participants, emergency evacuations such as for hurricanes, disasters, or when the Governor declares weather or other emergency.
- If an emergency arises where the staff must accompany the participant to receive medical attention.
- If the ADHC is transporting the participant to a medical appointment, then a four (4) hour stay is acceptable.

These cases must be approved by the CLTC Area Office and will be addressed on a case-by-case basis upon receipt of a request describing the circumstances. The Area Administrator or Designee will enter a narrative note indicating request and/or approval.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in the order of preference. These choices may be obtained verbally, by mail or during an in-home visit. If the choices are made verbally, the choices are documented in the narrative in order of preference.

The **Case Manager** will update the Service Plan to include the need for Adult Day Health Care.

Prior approval must be requested via the service request section of the Service Plan (under the Complete tab) in Phoenix.

Service Justification: Emphasize socialization needs and/or caregiver relief and/or need for supervision, etc.

The **Case Manager** will create a referral to the chosen providers in order of preference via Phoenix.

If the referral is accepted, the **Case Manager** notifies the ADHC of the pertinent participant information including the contact information for the physician. The ADHC must obtain the physician orders for the participant to attend ADHC utilizing the DHHS Adult Day Health Care Form 122DC. The ADHC can locate this form in the Phoenix provider portal. Once the ADHC has obtained the physician's order, the ADHC must notify the **Case Manager** to negotiate the start date for services to begin.

The **Case Manager** will authorize the amount, frequency, and duration of services in accordance with the participant's Service Plan.

The Service Provision Form must be sent to the provider via Phoenix to initiate services. The initial Service Plan and the new Service Plan created following re-evaluation will be available to the provider via Phoenix.

If a participant transfers from one ADHC provider to another, the participant's existing Physician order for ADHC remains effective only if it is current and completed within the last two years. The transferring facility will send a copy of the Physician order to the accepting facility.

7B Adult Day Health Care-Nursing

One (1) unit of service includes any one or more of the allowed ADHC-Nursing Services during one (1) day's attendance at the ADHC.

This service is provided by a licensed nurse in the ADHC. ADHC-Nursing is limited to the following skilled procedures:

- Ostomy care
- Urinary catheter care
- Decubitus ulcer/wound care
- Tracheostomy care
- Tube feeding
- Nebulizer treatment that requires medication

The **Case Manager** will update the Service Plan to include the need for Adult Day Health Care-Nursing.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Note: An intervention for at least one of the skilled procedures must be included on the Service Plan in order to request prior approval.

Service justification: State skilled procedure(s) that are requested for MD to order. (Note: These may differ from what the MD actually orders but allows the referral to go forward. Any differences on signed MD Order as shared by ADHC Nurse should be narrated.)

The **Case Manager** will create a referral to the chosen ADHC provider.

The ADHC provider must obtain the physician orders utilizing DHHS Form 122DC, prior to the initiation of the service, for the participant to receive ADHC Nursing. The ADHC is responsible for obtaining the initial order as well as the direct care and ongoing orders including changes in frequency. Once the ADHC has obtained the physician's orders, the ADHC must notify the **Case Manager** to negotiate the start date for services to begin.

The **Case Manager** will authorize the amount, frequency, and duration of services in accordance with the participant's Service Plan. The authorization is not day specific. There is one (1) procedure code for ADHC-Nursing regardless of the procedure to be completed.

The authorization should include the number of units per week.

An authorization change is not necessary for a change in the days of the week the procedure will be provided unless a change in the frequency is needed.

If there is a change in frequency, the ADHC will obtain updated physician's order and will notify the **Case Manager**. The **Case Manager** will issue a new Service Provision Form, if necessary. For example, the Procedure was ordered to be done two (2) times per week and is now needed five (5) times per week.

If there is a change in the Procedure, the ADHC will notify the **Case Manager**. The **Case Manager** will document the contact with the provider in the narrative or through the Conversation section in Phoenix. The **Case Manager** will update the Service Plan if indicated. A new Service Provision Form is not needed. For example, the participant was receiving nebulizer treatments and now urinary catheter care was added. The change would require documentation of the contact and an update in the Service Plan but would not require a new Service Provision Form.

The Service Provision Form must be sent to the provider via Phoenix to initiate services. The initial Service Plan and the new Service Plan created following re-evaluation will be available to the provider via Phoenix.

7C Attendant Care

The unit of service is one (1) hour. This service may be authorized in one-half (1/2) hour increments after a minimum of one (1) hour.

The objectives of the Attendant Care Service are to restore, maintain, and promote the health status of Medicaid Home and Community Based waiver participants through home support, medical monitoring, and assistance with activities of daily living.

Attendant Care is a service to assist with ADL and IADL care. This service is self-directed by the participant or directed by someone who is able and willing to be the Employer of Record (EOR) for Attendant services.

The Attendant must demonstrate an ability to read, write and communicate effectively with the Participant and/or their Representative.

No Physician Order is required for this service.

Prior Approval is required before sending a referral to UAP. If Attendant Care will replace existing agency personal care services, this will be indicated in the prior approval request.

Service Justification: Include a condition or conditions affecting participant's ability to perform ADL's and IADL's. (Examples: Shortness of breath related to COPD; unsteady on feet due to diabetic neuropathy; frailty due to advanced age). Include circumstances that may indicate need for high or increased service levels, such as recent hospitalization, physical decline of participant or caregiver, caregiver's need for respite, or loss of caregiver. **Note: Do not include number of hours in the comments on the service plan or desired weekly schedule.**

Note: When Attendant Care is replacing existing agency personal care services, the authorization for the agency services will remain in effect until the Attendant Care start date. The agency personal care services will be terminated effective the day prior to the start date of Attendant Care.

If the Attendant is unable to or fails to check-in or check-out when providing service, the Attendant must complete a Resolution Log acknowledging the service was provided and the reason for the resolution. Refer to the 12 Strike Policy in Phoenix Help for additional requirements.

Participants must be present while services are being provided.

Attendant Care activities include:

- a. Support of activities of daily living, e.g., assistance with bathing, dressing, feeding, personal grooming, personal hygiene, transferring and mobility;
- b. Meal or snack preparation, planning and serving, cleaning up afterwards, following specially prescribed diets as necessary and encouraging participants to adhere to any specially prescribed diets;
- c. General housekeeping includes cleaning (such as sweeping, vacuuming, mopping, dusting, taking out the trash, changing bed linens, defrosting and cleaning the refrigerator, cleaning the stove or oven, cleaning bathrooms) and activities as needed to maintain the participant in a safe and sanitary environment. Housekeeping only includes areas specific to the participant such as the participant's bedroom, bathroom, etc.
- d. Shopping assistance, essential errands, and escorting participant to medical services.
Attendants are not allowed to provide transportation.
- e. Assistance with communication, which includes, but is not limited to placing a phone within participant's reach and physically assisting participant with the use of the phone, and orientation to daily events.
- f. Monitoring medication, e.g., the type that would consist of informing the participant it is time to take medication as prescribed by the physician and as written directions on the box or bottle indicate. It does not mean that the attendant is responsible for giving the medicine; however, it does not preclude the attendant from handing the medicine container or medicines already set up in daily containers to the participant.

Skilled Services Provided by an Approved Attendant

The waiver service definition for "Attendant" allows a participant or the designee to self-direct "skilled service activities" performed by the Attendant with approval from the participant's physician. These activities are considered "health maintenance activities" which could be performed by the participant if the individual were physically and mentally capable. With Physician approval, Attendants may provide skilled services which are considered health maintenance activities for participants in all Community Long Term Care Home and Community Based Services Waivers.

When a referral is being made to UAP for Attendant Care and it is being requested the Attendant provide Skilled Service Activities, the Case Manager will include a comment on the referral stating “Participant/Authorized Representative has requested a skilled service to be provided.”

For existing referrals, already in process with UAP, where a skilled service need has been identified, the Case Manager will note in the comment section on the referral, “Participant/Authorized Representative has requested a skilled service to be provided.” AND email UAP (UAP@scdhhs.gov) to notify them of the change in the referral. UAP will not receive a Phoenix notification that there is a change to the referral.

UAP will send the “MEDICAID HOME COMMUNITY BASED WAIVERS PHYSICIAN STATEMENT FOR PARTICIPANT DIRECTED ATTENDANT CARE” and a copy of the legislation to the assigned Case Manager, who will provide the form and a copy of the legislation to the Participant or Attendant’s Employer of Record. The Participant or Employer of Record or Designee is responsible for obtaining the physician approval/signature.

Once the signed Physician’s Statement has been received, the Case Manager will scan the form into Phoenix scans under the Attendant scan tag and email UAP to notify the form has been returned and scanned. UAP will complete the Attendant match visit once this notification is received by email.

The following list of services are examples of activities which may be appropriate for the Attendant to perform. *This list is not all inclusive.*

- Tracheostomy care
- Ventilator Care - taking on and off the vent
- Suctioning- tracheostomy/ oral
- Finger stick blood glucose monitoring
- Wound care
- Enteral feedings (PEG or Nasogastric)
- Bowel program
- Ostomy care
- Catheter care

Any other skilled service requested by the participant/Employer of Record/Designee or physician requires Central Office approval.

NOTE: *If the participant changes physicians* and the physician who signed the Physician Statement is no longer involved in the participant's care, a new Physician's Statement is required. If the service is directed by the Employer of Record (EOR) instead of the participant and the Employer of Record is no longer able to direct the skilled service activities performed by the Attendant, a new physician statement is required.

Discussion & Selection of an Employer of Record (EOR) for Attendant Care Services

If the **Case Manager** and participant and/or Authorized Representative identify Attendant Care as a need, the **Case Manager** will discuss the need to identify an Employer of Record (EOR). For the **participant** to be approved for Attendant Care services, the participant or designee will serve as the Employer of Record (EOR). The EOR for attendant service must be knowledgeable and involved in the participant's day to day care.

The Authorized Representative cannot serve as the Attendant unless there is an approved designated Employer of Record (EOR) who assumes the responsibility of all attendant care related contacts with the Case Manager.

The EOR should reside within a 50-mile radius of the participant's home. Any exceptions must be approved by UAP and **Waiver Service Exceptions**. **Waiver Service Exceptions requests should be emailed to WES@scdhhs.gov.**

Steps to request an exception for EOR over 50 miles:

1. Case Manager narrate the EOR will be over 50 miles away.
2. Case Manager confirm and narrate that the EOR is the only person available to perform the task and has agreed to do so and that the EOR understands and has agreed to assume the roles and responsibilities.
3. Documentation must be present that the Area Administrator or Lead Team agrees with the justification and gives approval for the request to be sent to Waiver Service Exceptions for review.

The Employer of Record (EOR)

- Must be willing and able to interview prospective Attendants;
- Meet with UAP in the home for all match visits and when follow-up visits are necessary;
- Discuss the participant's attendant care needs by telephone with the appropriate CLTC staff or **Case Manager**;

- Meet with the **Case Manager** in the participant's home for the Initial Visit if available;
- Meet with the **Case Manager** in the participant's home for quarterly visits if available;
- Meet with the **Case Manager** in the participant's home for annual re-evaluation visits;
- Attend the initial Attendant/EOR Care Call training if available.
- Must provide regular supervision of the Attendant;
- Must provide guidance, direction and assistance with problems related to the participant's care to the Attendant;
- Must review and sign the Attendant Logs at least monthly, acknowledging that services were or were not provided by the attendant on the days notated on the logs;
- Must be present to observe the care provided by the Attendant at least monthly;
- Must review and sign the Resolution Form completed by Attendant, acknowledging that the service was provided if the Attendant is unable to or fails to check-in or check-out on the designated landline phone or Mobile App.
- **The signed Resolution Log must be received by the CLTC Area Office within thirty (30) days of the date of the Attendant Care service or a strike will be issued.**
- The CLTC Area Office **Support Staff** will work resolutions within 10 business days of submission.

Note: When a vision impaired participant self-directs Attendant Care, UAP initially reads related forms to the Participant and instructs Attendants to read the Daily Logs to the Participant, prior to signing.

Referrals to University Affiliated Program (UAP) for Attendant Care

Once a prospective EOR is identified, and the prospective EOR has identified a prospective Attendant, the prospective EOR will discuss the choice of provider with the participant and **Case Manager**. If the **Case Manager** agrees that this is a reasonable option for participant care, the **Case Manager** will:

- Document the choice of provider.
- Complete a referral to UAP in Phoenix. Choose "Approval Request" on the carousel to complete the appropriate sections of "Participant Directed Care" or

“Representative Directed Care”. The **Case Manager** should alert UAP of any concerns when sending the form to UAP, such as APS involvement, etc. in the comment section.

- UAP sends an information packet to the EOR and an enrollment packet to the Attendant. An UAP Nurse contacts the participant’s prospective EOR to discuss Attendant Care service.
- UAP will make arrangements to visit the participant and the prospective EOR & Attendant in the participant’s home after the Attendant meets the requirements to enroll. Match Visits by UAP must include observation of the ADL care as outlined in the service plan.
- Upon completion of a match visit, UAP will notify the **Case Manager** by approving the match via Phoenix.
- **Case Manager** will review any comments provided by UAP at the Match Visit.
- After receiving notification of approval in Phoenix from UAP, the **Case Manager** will check the resource directory to verify the **Attendant’s** enrollment and review the first and/or second light(s) are green in Phoenix. If enrolled and the appropriate lights are green in Phoenix, the Area Office Attendant Care Trainer determines if training is necessary. Refer to the CLTC Standardized Attendant Care Training Manual for additional guidance.

Note: If Care Call training has been attended by the chosen provider within the past 6 months, it is not necessary to repeat the training unless a refresher session is requested. Care Call training may need to be repeated if the Attendant has not provided services in the last 6 months.

- If the Attendant has not been enrolled, an enrollment notification will be sent from Central Office when the provider has been enrolled and is in the Resource Directory.
- When the provider has been enrolled, **Attendant Care Trainer** sends an e-mail to the **Case Manager** to notify of the status of the Attendant’s enrollment.
- UAP instructs the Attendant to contact the **Case Manager** to schedule Care Call Training after the Attendant receives a provider number.
- If Phoenix shows the red stop light under “ok to authorize” Attendant services, **do not authorize the service**. View status screen to determine the enrollment information that is missing.
- **Case Manager** reviews the specific attendant information in the Participant’s Phoenix case in the Attendant Care Request section.

- If Phoenix shows the green light under “ok to authorize” Attendant services, and Care Call training is needed; the CLTC Trainer should arrange Attendant/EOR Care Call Training.
- The Case Manager Authorizes the total number of hours per week. The EOR and Attendant will negotiate the appropriate days and times of service to meet the participant’s needs as outlined by the Participant’s Service Plan.
- The **Case Manager** reviews the service plan and duties with the Attendant during the Care Call Training.
- The **Case Manager** reviews the authorization with the Attendant. Unless otherwise approved by the **Area Administrator** or **Lead Team**, Attendant Care services will be started with the following Sunday’s date. The Service Provision Form designates the units per week, not specific days.
- The **Case Manager** provides the attendant with a Service Provision Form.
- The **Case Manager** provides the attendant with a copy of the Service Plan.

When Attendant Care service is terminated, the Case Manager:

- Sends a copy of the service termination form to the Attendant provider.
- Contacts the Attendant provider by phone if the termination is effective immediately.
- Notifies UAP of the termination of the service.
- Updates the Service Plan to reflect the change in service.
- Terminates the authorization when/if the attendant is terminated.
- Completes a Notice of Discontinued Employment Form and faxes to Financial Management Service.
- The Notice of Discontinued Employment Form is only to be completed when the Attendant is fired and never going to be requested back for employment. A copy of the Notice of Discontinued Employment Form should be uploaded in Phoenix and narrated when it was sent to Financial Management Service.

When there is a request to change Attendant Care Provider choice, the Case Manager:

- Receives and discusses the selection of the new prospective Attendant with the EOR.
- Complete a new pre-screen under the Approval Request tab in Phoenix.

- Awaits UAP approval of the match with the new Attendant in Phoenix.
- Works with **CLTC staff in Area Office who provide Attendant training** to schedule training, if needed.
- If Phoenix shows the green light under “ok to authorize” Attendant services, and Care Call training is needed; the CLTC Trainer should arrange Attendant/EOR Care Call Training.
- Authorizes the total number of hours per week. The EOR and Attendant will negotiate the appropriate days and times of service to meet the participant’s needs as outlined by the Participant’s Service Plan.

When more than one Attendant is authorized for a participant

When more than one Attendant provides services on a regular basis, all Attendants will be authorized the total amount of Attendant Care hours approved to provide services on an ongoing basis. A comment is included on the authorization stating the total combined Attendant hours cannot exceed the total hours approved. For example: If fifteen (15) units authorized between two (2) Attendants and primary Attendant provided twelve (12) hours; this leaves only three (3) units for the second (2nd) Attendant. The EOR must closely work with the two (2) Attendants regarding the specific times that each will provide service since the total hours cannot exceed fifteen (15). The **Case Manager** must monitor hours closely to ensure the fifteen (15) hour total is not exceeded.

Exception: If both attendants provide a specific number of units each week, the hours can be divided and authorized specific to the EOR’s designation of the break-out of hours. The comment section will be used to communicate this information to the provider.

If only a one-time/temporary backup Attendant is necessary, the service is authorized at the time of the usage. The EOR or Attendant should notify the **Case Manager** in advance, if possible. If the one-time/temporary backup Attendant is necessary when the CLTC Area Office is closed and unable to provide prior approval, the EOR must call as soon as the CLTC office reopens. The authorization must specify the number of hours worked and have a start and end date. The primary authorization is left open. The Attendant should check-in and check-out in Care Call. It is the **Case Manager’s** responsibility to assure the approved number of hours are not exceeded.

Case Manager On-going Responsibilities:

1. Daily Logs

- The Daily Attendant Log is an important documentation tool used by all Attendants to record services provided.

The log is to be completed daily by the Attendant outlining only the specific tasks performed on the noted date as part of the participant's care. The log is to be signed weekly by the EOR and to be received in the CLTC Area Office that serves the participant by the fifteenth (15th) of the following month. The CLTC **Support Staff** will receive the attendant logs and securely forward to the assigned **Case Manager**.

- The assigned **Case Manager** will review all information on the log to ensure that the Attendant Care services notated as provided to the Participant are in compliance with the Participant's Service Plan and match the Care Call information through Phoenix. The **Case Manager** should also make sure the logs have the required signature of the Attendant and the EOR and that they are not copies of previously submitted logs. **The Case Manager is responsible for uploading the daily logs into Phoenix to the appropriate "Attendant" scan tag.**
- Receipt and review of the logs, as well as any Attendant comments showing problems or changes in the condition of the participant, should be narrated in Phoenix **no later than the twenty-second (22nd) day of the month.** The completed logs will be scanned into Phoenix scans under the "Attendant" scan tag. If the logs are not received with appropriate signatures by both the Attendant and Employer of Record by the fifteenth (15th) of the month, the **Case Manager** will complete and mail a CLTC Notification Form to the Participant/Authorized Representative and EOR no later than the twenty-second (22nd) of that month. The termination date (often (10) business days from the date mailed) and reason for the termination is to be included in the CLTC Notification Form. If the logs are received by the deadline, the Service Provision Form will remain open.
- Should the Attendant fail to properly submit the logs within the time frame a second time for the same participant, the Attendant service for that Participant will be terminated and not reinstated for that participant/attendant match. The **Case Manager** should offer other services to the participant/Authorized Representative to meet the participant's service needs.
- If the logs are not received with proper signature by the end of the ten (10) business day notice, no further extension is required by the **Case Manager**. The attendant service is immediately terminated.
- If the Attendant is serving another participant, and is timely for that Participant, services for the second Participant may continue.
- Discrepancies or differences in services provided and those indicated on the service plan, Attendant Care Logs, or in Care Call should be investigated and resolved by the **Case Manager**.

2. If participant or EOR is suspected of no longer being capable of supervision, UAP will be informed by the **Case Manager**.
3. If the EOR dies or is no longer able or willing to provide weekly supervision, the Attendant Service must be immediately terminated. In this situation, the participant/Authorized Representative and the Attendant are to be notified by phone of the termination. A copy of the termination form is mailed to the Attendant.
4. Any changes in the EOR must be referred as a new match visit to determine the prospective EOR's ability to direct care and to establish the Attendant as the Employee of the EOR with the fiscal agent.
5. **Case Manager** will notify UAP of any changes in the participant's condition or situation that may impact the Attendant Care services. (Example: Attendant is no longer providing care to participant).
6. The CLTC Area Office Representative **will** receive written Care Call Resolution Forms and will contact the **Case Manager** for confirmation that the service was provided. All requests for confirmation of resolutions should be reviewed and investigated, and the **Case Manager** must respond to the **CLTC Area Office Representative** for approval/denial within seven (7) business days. Any delay in response back to **the CLTC Area Office Representative** should be narrated, giving reason for the delay. Prompt action to finalize the resolution is expected.
7. If UAP inactivates a match and the attendant is suspended or terminated, UAP will inform the assigned **Case Manager**. The **Case Manager** will notify the participant, attendant and EOR of the suspension or termination. The **Case Manager** will terminate the authorization. UAP will confirm the termination and will contact the **Lead Team Case Manager** or **Area Administrator**, if needed.

Monitoring of Service by UAP

Problem resolution visits by the **UAP Nurse** may be requested by the participant, Authorized Representative, EOR, Attendant, **Case Manager**, **Lead Team Case Manager** or **Area Administrator**. All appropriate parties will be consulted for problem resolution.

Service level approvals are time limited until the next re-evaluation or as indicated by the Reviewer in approval comments.

7D Handheld Shower

One (1) unit of service includes one Handheld Shower. One unit allowed per year.

Handheld Shower must be installed/adjusted by the provider. Items may not be dropped shipped to the participant.

The need for this equipment must be evaluated in the home prior to authorizing the service. The need for this service must be documented in the Home Assessment section of Phoenix.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in order of preference. These choices may be obtained verbally, by mail or during an in-home visit. If the choices are made verbally, the choices are documented in the narrative in order of preference.

The **Case Manager** will update the Service Plan to include the need for Handheld Shower.

Prior approval must be requested via the service request section of the Service Plan in Phoenix. The **Case Manager** will create a referral to the chosen provider in order of preference via Phoenix.

Service Justification: Handheld Shower: Focus on weakness, poor endurance. Include date last item was received.

No Physician Order is required for this service.

The **Case Manager** will authorize the services in accordance with the participant's Service Plan. This Service Provision Form will include a start date and an end date which will be thirty (30) days later.

The Service Provision Form must be sent to the provider via Phoenix to initiate services. The Service Plan is not required to be sent to the provider for this service.

The cost of Handheld Shower is not included in the \$7,500.00 lifetime cap for Environmental Modification services.

CLTC STANDARDS FOR HANDHELD SHOWER

Specs for Handheld Shower:

Minimum 6' hose
Pause/shut off function
Installation with Teflon tape

7E Case Management

Case management is a waiver service that is provided to all participants who participate in the CLTC program. It is the process of discussing and coordinating services to enable the participant to remain in the home. Case management begins on the date the participant enters the program and continues through the date of application closure. It is a vital part of the long-term care service system. Some participants may choose and/or need only case management services. In such situations, the value of case management is in the continued follow-up, coordination, and monitoring of services other than CLTC waiver services. On-going case management enables the **Case Manager** to evaluate changes in the participant's service needs.

A unit of service is one (1) month of Case Management.

The intensity of Case Management services provided to each participant is dependent upon the individual participant's needs as set forth in the participant's Service Plan.

The Service Plan will include the Case Management Service.

Prior approval must be requested via the service request section of the Service Plan in Phoenix. A written justification reason is not required. Prior approval will be obtained for one (1) unit of Case Management monthly.

At a minimum, case management requires a participant contact monthly. The contact types include Initial Visit, Monthly Contact, Quarterly Visit and Re-evaluation Visit. One of these contact types must be provided and documented every month.

A referral is created and sent to the chosen providers in order of preference via Phoenix.

Service Justification: No justification needed.

If the referral is accepted, the **on-going Case Manager** has forty-eight (48) business hours to contact the **Case Manager II** to receive the case. The case will be discussed with the **on-going Case Manager**. The **Case Manager II** will complete the Service Provision Form for Case Management. Documentation of the case referral and acceptance must be made in Phoenix by the Case Manager II.

Case Management Services must be provided Monday through Friday between the hours of 7:00am- 7:00pm. If the participant and/or Authorized Representative request case management contact or visit outside of these hours for the participant's benefit, approval must be received from and narrated in Phoenix by the **Area Administrator**.

Case Management shall include the following activities:

- A. Person Centered Service Planning
- B. Service Coordination
- C. Monitoring
- D. Re-evaluation
- E. Service Plan Evaluation
- F. Service Counseling
- G. Initial Authorization of Waiver Service
- H. Changes in Services within an Authorization Period
- I. Termination of Waiver Services
- J. Application or Case Termination and/or Transfer
- K. Documentation of each activity reflecting date of occurrence.

Note: Case Management providers should refer to other related chapters as well as the requirements of the Community Long Term Care Case Management Contract and the Case Management Scope of Services at:

<https://www.scdhhs.gov/provider-type/community-long-term-care-provider-manual-070119-edition-posted-070119>

7F Companion Services

The unit of service is one (1) hour (not including time for transportation to and from participant's residence, breaks or meals). This service may be authorized in one-half (½) hour increments after a minimum of one (1) hour.

Companion services are provided in the participant's residence and are intended to provide relief for caregivers with needed supervision of participants. **The service does not include hands on care.**

Participants must be present when this service is being provided. Companion activities include:

1. Socialization – reading, conversation, assistance with participant's mail or newspaper;
2. Assistance with meal or snack preparations, serving and cleaning up afterwards;
3. Incidental housekeeping including sweeping, dusting, assistance with or supervision of the participant's laundry or linens, and other light chores needed to maintain the participant in a safe and sanitary environment. Housekeeping is only for the areas specific to the participant such as the participant's bedroom, bathroom, etc.
4. Shopping assistance, essential errands, and escorting participant to medical services. **Companions are not allowed to provide transportation.**
5. Sitting service focusing on the participant including supervision, orientation, and observation for basic safety to include making emergency contact if needed.

Companion services can be provided by an agency provider or Medicaid enrolled individual companion provider.

A person who is the primary caregiver cannot be paid to provide companion services. A primary caregiver is someone who provides hands-on care as a natural support to the participant. There could be more than one primary caregiver identified. The reason for this is the service is primarily designed to give relief to the primary caregiver(s).

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in the order of preference. These choices may be obtained verbally, by mail or during an in-home visit. If the choices are made verbally, the choices are documented in the narrative in order of preference.

The **Case Manager** will update the Service Plan to include the need for Companion Service.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Service Justification: Agency or Individual: State type of activity or activities needed, such as socialization, supervision, respite for caregiver, essential errands, etc. **Note: Do not include number of hours, desired time of day, or desired days of week/weekly schedule.**

The **Case Manager** will create a referral to the chosen providers in order of preference via Phoenix.

If the referral is accepted, the **Case Manager** will negotiate the start date for services to begin.

No Physician Order is required.

The **Case Manager** will authorize the amount, frequency, and duration of services in accordance with the participant's Service Plan.

The Service Provision Form must be completed. The duties of the Companion- Individual must be listed in the Comments section of the Service Provision Form. The Service Provision Form must be given to the Companion- Individual by mail or hand delivery.

The Service Provision Form must be sent to the Companion Agency via Phoenix. The Service Plan is available to the provider in Phoenix.

Service level approvals are time limited until the next re-evaluation or as indicated by the Reviewer in approval comments.

Companion Services Provided by Individual Companion Providers

The Companion Individual Service is authorized by the week.

If the **Case Manager** and participant/Authorized Representative identify Companion (Individual) as a need, then the **Case Manager** will:

- Complete the Pre-screening in Phoenix for Participant Directed Care (UAP Form 1) OR Pre-screening for Assessment for Representative Directed Care (UAP Form 2).
- Obtain a Choice of Provider.
- Make a referral to UAP if the pre-screening is passed. The referral is made by emailing UAP the participant's name and Client ID.

- UAP conducts the enrollment review process (PPD, SLED check, Fiscal Agent paperwork) and notifies the **Case Manager** of the findings.
- UAP notifies the participant and/or Authorized Representative of the SLED check results.
- If approved by UAP, monitor Phoenix for approval and for the CLTC Central Office notification of enrollment.

The **CLTC Area Office Support Staff** will:

- Check the Resource Directory to verify companion's enrollment as a provider after receiving email from UAP.
- Verify if the training box has been completed if the companion is enrolled. If the companion has not been enrolled, an enrollment notification will be sent from Central Office when the provider has been enrolled and is on the Resource Directory.
- Send an email notification to the **Case Manager** when the companion is enrolled.

The **Case Manager** will then review the specific Individual Companion information in the Participant's Phoenix record under the Attendant Care Request section.

- If the light under "ok to authorize" for companion services in Phoenix is red, **do not** authorize the individual companion services. View status screen to determine what enrollment information is missing.
- When the light under "ok to authorize" companion services in Phoenix is green, proceed to arrange billing training for the provider in the CLTC Area Office with the **CLTC Area Office Attendant Care Trainer**. During training, the **Case Manager** to review the activities to be performed by the Companion.

Note: If billing training has already been attended by the chosen provider, it is not necessary to repeat the training. The service authorization including the activities to be completed in the comments section, may be mailed to the companion provider.

- Completes Service Provision Form in Phoenix and sends a copy to the provider and UAP.

- The Companion does NOT complete Daily Logs.
- Completes a service termination in Phoenix when the service is terminated and sends a service termination to the companion provider. Also, UAP must be notified of the termination date of the companion service. The Service Plan must be updated accordingly.
- Completes a Notice of Discontinued Employment Form and faxes to Financial Management Services

After the Companion service starts: The **Case Manager** should make every effort to schedule the next quarterly home visit during a time when the companion provider is available. The **Case Manager** should review the service expectations with the companion and participant and/or Authorized Representative and monitor for quality and appropriateness.

Companion Services Provided Through an Agency

The Companion Agency Service is authorized by the day.

Service Provision Form must be sent to the provider designating the units per day. It is important that the **Case Manager** coordinate this service with other services going into the home as well as with the companion's supervisor.

7G Environmental Modifications

This service must be participant specific and for the benefit of the participant. Environmental Modification must be necessary to ensure health, welfare and safety and enable the participant to function with greater independence in the home without which, the participant would require institutionalization.

The service must be cost effective. The **Case Manager** must exercise professional judgment regarding the appropriateness of the service. The service is not intended for major home improvement, remodeling, or cosmetic improvements. The **Case Manager** must explore other available community resources prior to requesting approval for this service.

The need for Environmental Modifications including bath safety equipment must be evaluated in the home by the Case Manager prior to requesting approval. The need for this service must be documented in the Home Assessment section of Phoenix. During the home visit, the **Case Manager** must explain and obtain appropriate signatures on the Participant or Landowner Consent Form for all the environmental modification services.

Exception: Authorization of fans or air conditioners does not require a Participant or Landowner Consent Form. A home visit is not required for fans, or air conditioners. A phone call will be acceptable for these items only.

The **Case Manager** will update the Service Plan to include the need for Environmental Modification.

Prior approval must be requested via the service request section of the Service Plan in Phoenix. The service prior approval is required prior to sending the request to the Environmental Modification Specialist.

Note: There is a lifetime cap of \$7,500. This includes the costs for ramps and enhanced environmental modifications. Requests to exceed this amount must be sent to **Central Office** for consideration and approval.

7G-1 Fans/Air Conditioners

One (1) unit of service includes one of the allowed items of Fans or Air Conditioners.

The need for this equipment can be evaluated and assessed by a phone call. A home visit is not required for fans or air conditioners. The need for this service must be documented in the Home Assessment section of Phoenix.

One (1) Fan and one (1) Air Conditioner can be authorized every two (2) years, if needed. No exception shall be granted.

The Participant or Landowner Consent Form is not required for this service.

The **Case Manager** will update the Service Plan to include the need for Fan/Air Conditioner.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Service Justification: Emphasize need based on health and safety related issues. Include date last item was received.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in order of preference. These choices may be obtained verbally, by mail or during an in-home visit. If the choices are made verbally, the choices are documented in the narrative in the order of preference.

The **Case Manager** will create a referral to the chosen providers in order of preference via Phoenix.

The **Case Manager** will authorize the amount, frequency, and duration of service in accordance with the participant Service Plan. This Service Provision Form will include a start date and an end date which will be thirty (30) days later.

The modification specifics must be clearly stated:

- Air conditioners must be 10,000 BTU. This unit includes installation and a one-year in-home warranty. During this one-year warranty period, it is the responsibility of the provider to replace and/or repair the unit. The unit must be present at the home and not discarded.
- Box fans must have three (3) speeds.

The **Case Manager** must verify service and satisfaction with the work completed by the provider, as appropriate, on the next scheduled contact after the authorization end date. If there are problems, the **Case Manager** shall negotiate resolution of the problems and/or complaint with the provider.

7G-2 Ramps

This policy is to be used when a ramp is needed, and no other Environmental Modifications are needed.

The need for this service must be evaluated in the home prior to authorizing the service. The **Case Manager** will visit participant's home to assess the need for a ramp. This visit may be completed at the next regularly scheduled home visit unless there is a critical need requiring an earlier visit.

Only one ramp may be authorized at a time for a participant. If a participant changes location and requests a new ramp or a participant's current ramp is deemed unsafe, the Case Manager may make a home visit to assess need. An additional ramp may be approved if the participant has not exceeded the \$7500 lifetime cap.

The need for this service must be documented in the Home Assessment section of Phoenix.

During the home visit, the **Case Manager** must explain and obtain appropriate signatures on the Participant or Landowner Consent Form. The consent form signature is to be obtained in Phoenix. If a hard copy form is used, the signed form must be scanned into Phoenix under the Environmental Modification Scan Tag.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain provider choice(s) from the participant and/or Authorized Representative in order of preference. These choices may be obtained verbally, by mail or during an in-home visit. If the choices are made verbally, the **Case Manager** will document the choices in the narrative, in the order of preference. The participant and/or Authorized Representative should be encouraged to contact chosen provider(s) to determine if the provider is willing to accept the referral and to determine how long it will take the provider to complete the work.

Under no circumstances shall a **Case Manager** assist the participant in choosing a provider.

The **Case Manager** will update the Service Plan to include the need for a ramp.

Prior approval must be requested via the "New Ramp Request" section from the Home Assessment screen in Phoenix. Answering the ramp request questions in Phoenix regarding the door, yard size, and home elevation will determine whether the ramp request is referred to the Ramp Provider or Environmental Modification Specialist.

Prior to submitting pictures in the Phoenix ramp workflow, make sure of the following:

- Pictures need to be at a wide angle to show where the new ramp will need to be placed.
- Take the pictures from the street and include the house and yard in the pictures. Pictures that are taken too close to the house are not acceptable.
- Include multiple angles of the new ramp location in the pictures.

When taking threshold measurements, measure in inches from the door threshold to the **landing**. This is usually only 1”-3” to the outside structure (landing, porch, steps). Measurements from the door threshold to the **ground** are not acceptable. Refer to the Ramp Referrals – Case Manager guidance in Phoenix Help for more information.

Service Justification: State need based on mobility, accessibility, and/or health and safety issues. State Participant or Landowner Consent Form was signed, and the document is in Forms or has been scanned. If ramp was provided at a previous address, include date. Include amount currently spent to date towards lifetime cap.

The **Case Manager** will complete the ramp form in the Home Assessment section in Phoenix. Click on “Refer to Providers”. Notate if the Environmental Modifications Specialist should review “difficult situation” such as homes that are elevated and high off the ground, homes with small yards and when there is a discrepancy about which door is to be used.

The **Case Manager** will create a referral to the chosen provider in the order of preferences via Phoenix.

Note: Do not put an end date on this referral. Send the referral from the Home Assessment section in Phoenix. Complete all fields of the referral.

When the referral is accepted, the **Case Manager** will receive notification in the Notification section in Phoenix. No further action is needed by the **Case Manager** until the provider submits the Ramp Completion Form via Phoenix.

When the ramp is completed, the provider must submit the Ramp Completion Form in Phoenix. Upon notification of receipt of the Ramp Notification Form via Phoenix, the **Case Manager** must complete the Service Provision Form. The start date is the date the Service Provision Form is created, and the end date is thirty (30) days later. The dollar amount to be entered on the Service Provision Form is the dollar amount indicated by the provider via the Ramp Completion Form.

If a ramp and an Enhanced Environmental Modification are being requested at the same time for the same participant, see Chapter 7, Section 7H-4.

If the participant and/or Authorized Representative is dissatisfied or complains about the ramp and an inspection by the Environmental Modification Specialist is needed, the **Case Manager** will email EVIR to request an inspection. **The Case Manager will proceed with creating and sending the Service Provision Form to the provider while the Environmental Modification Specialist is reviewing the complaint. Provider payment should not be delayed pending review by the Environmental Modification Specialist.**

All charges in the “other” category must be pre-approved by the Environmental Modification Specialist.

It is the responsibility of the provider to construct all jobs to the required code of the Americans with Disabilities Act (ADA), as well as meeting specific requirements stipulated by CLTC. In addition, all required permits are the responsibility of the provider.

Participants are responsible for ramp maintenance. The Case Manager may request ramp inspection by the Environmental Modification Specialist if a ramp is deemed unsafe or the structure is compromised. The Environmental Modification Specialist will determine whether or not to approve ramp repair or replacement.

The cost of Ramps service is included in the \$7,500.00 lifetime cap for Environmental Modification services.

7G-3 Enhanced Environmental Modifications

Enhanced Environmental Modifications needs may be requested for participants who own their own home/property, or the home is owned by a family member where no rent is paid to the family member. Rent to own is considered renting and requires documentation.

The need for this service must be evaluated in the home prior to authorizing the service. The **Case Manager** will visit participant's home to assess the need for the Enhanced Environmental Modifications. The need for this service must be documented in the Home Assessment section of Phoenix.

During the home visit, the **Case Manager** must explain and obtain appropriate signatures on the Participant or Landowner Consent Form.

Prior approval must be requested via the service request section of the Service Plan in Phoenix. The service prior approval is required prior to sending the request to the Environmental Modification Specialist.

Service Justification: State need based on mobility, accessibility, and/or health and safety issues. State Participant or Landowner Consent Form was signed, and the document is in Forms or has been scanned. Include amount currently spent to date towards lifetime cap.

The **Case Manager** will complete the Environmental Needs Tab of the Home Assessment section in Phoenix. The **Case Manager** will indicate the type of Enhanced Environmental Modification needed from the list in Phoenix and will send the request to the Environmental Modification Specialist via Phoenix.

The Environmental Modification Specialist will assess the request and indicate via Phoenix when the request is ready for bidding.

The providers serving the geographical area of the participant will receive an email and a Phoenix Notification inviting them to bid on the Enhanced Environmental Modification. Providers will have access to details of the job including location and pictures in Phoenix.

Note: If the Environmental Modification Specialist specifies other waiver services to be provided by the environmental modification provider in the bid, the provider will not include the cost of these items in the bid amount. The Case Manager will create separate Service Provision Forms for each waiver service. For example, the environmental modification specialist may indicate in the bid proposal that the provider will provide a shower chair. The provider will not include the cost of the shower chair in the bid amount. **The Case Manager will create separate authorizations for the environmental modification and the shower chair.**

Once the bid period is closed, the Environmental Modification Specialist will award the job by notifying the winning bidder. The **Case Manager** will be notified via Phoenix Notifications.

The Enhanced Environmental Modification provider indicates via Phoenix that the Enhanced Environmental Modification project has been completed. The **Case Manager** will be notified of the awarded bid dollar amount and the provider information upon the project completion. The **Case Manager** will be able to see this information both at the point of bid award and at the point of job completion. The **Case Manager** must click on the notification to see the Enhanced Environmental Modification project screen where the awarded bid dollar amount is shown. The **Case Manager** will click “Create Authorization.”

The **Case Manager** will create a Service Provision Form via Phoenix. The start date will be the date the Service Provision Form is created, and the end date will be thirty (30) days later.

The Enhanced Environmental Modification provider will bill through Care Call.

If the participant and/or Authorized Representative is dissatisfied or complains about the Enhanced Environmental Modification and an inspection by the Environmental Modification Specialist is needed, the **Case Manager** will email EVIR to request an inspection. The **Case Manager** will proceed with creating and sending the Service Provision Specialist Form to the provider while the Environmental Modification Specialist is reviewing the complaint. **Provider payment should not be delayed pending review by the Environmental Modification Specialist.**

The cost of Enhanced Environmental Modifications service is included in the \$7,500.00 lifetime cap for Environmental Modification services.

7G-4 Enhanced Environmental Modifications and a Ramp

Enhanced Environmental Modification may be requested for participants who:

- Own their own home/property or
- Live in a home which is owned by a family member where no rent is paid.
- Rental property can have a ramp, with landowner consent.

Assessment of Need

The **Case Manager** will visit participant's home to assess the need for Enhanced Environmental Modification and ramp.

This may be completed at the next regularly scheduled home visit. If there is an immediate need, the **Case Manager** will complete an in-home visit as soon as possible.

During the home visit, the **Case Manager** must explain and obtain appropriate signatures on the Participant Homeowner/Family Member Homeowner Consent Form.

The need for this service must be documented in the Home Assessment section of Phoenix.

Prior Approval

Prior approval for the environmental modification and the ramp must be requested separately in the service request section of the Service Plan in Phoenix. Prior approval is required prior to sending the request to the Environmental Modification Specialist.

Environmental Modification Specialist

The **Case Manager** will:

- Complete the Environmental Needs Tab of the Home Assessment section in Phoenix.
- Complete the ramp form in the Home Assessment section in Phoenix.
- Indicate the type of Enhanced Environmental Modification needed from the list in Phoenix.
- Send the request to the Environmental Modification Specialist via Phoenix.
- Wait for a response from the Environmental Modification Specialist through Phoenix before proceeding.

The **Environmental Modification Specialist** will assess the request and indicate via Phoenix when the request is ready for bidding.

Note: If there is a critical need for the ramp and the participant cannot wait for the bid process for the Enhanced Environmental Modification, the Case Manager will authorize the ramp in accordance with the participant's choice of provider and follow the process for the ramp service. Enhanced Environmental Modifications and the ramp can be provided by two different providers of choice. Both services do not have to be provided by the same provider.

Environmental Modification Provider Bidding Process

The providers serving the geographical area of the participant will:

- Receive email and Phoenix notification inviting them to bid on the enhanced environmental modification.
- Access to details of the job including location and pictures in Phoenix.
- Submit a bid via Phoenix within one week.

Once the bid period is closed, the **Environmental Modification Specialist** will award the job by notifying the winning bidder. The **Case Manager will be notified** via Phoenix Notifications.

The **Case Manager** must click on the notification to see the Enhanced Environmental Modification project screen where the awarded bid dollar amount is shown.

- If the **bid indicates the winning environmental modification provider must build the ramp**, the **Case Manager** will send a ramp referral to the winning environmental modification provider.
- If the **bid does not indicate the winning environmental modification provider must build the ramp**, the **Case Manager** will send a ramp referral to the ramp providers of participant's choice, in the order of preference.

Service Authorization for Environmental Modification

The **Enhanced Environmental Modification Provider** indicates via Phoenix when the project(s) is completed.

The **Case Manager** will:

- Click on the notification to see the Enhanced Environmental Modification project screen where the awarded bid dollar amount is shown.

- Click “Create Authorization.”

oCreate a Service Provision Form for the environmental modification. The start date will be the date of the Service Provision Form is created and the end date will be thirty (30) days later.

Service Authorization for Ramp

When the ramp is completed, the provider must submit the Ramp Completion Form in Phoenix.

If there are any additional items listed on the Ramp Completion Form under “other” column, the Ramp Completion Form will be sent to the Environmental Modification Specialist in Phoenix first for approval before going to the Case Manager for the authorization process.

The Case Manager will:

- Click on the notification of receipt of the Ramp Completion Form via Phoenix,
- Click “Create Authorization”
 - o Complete a Service Provision Form for the ramp. The start date is the date the Service Provision Form is created, and the end date is thirty (30) days later.
 - o The dollar amount of the Service Provision Form is the amount indicated by the provider via the Ramp Completion Form.

The provider will bill through Care Call.

The cost of Enhanced Environmental Modifications and a Ramp service is included in the \$7,500.00 lifetime cap for Environmental Modification services. (The lifetime cap is \$6,000.00 for environmental modifications).

7H Advanced Pest Control

If there is a known or suspected active bedbug infestation at the time of a scheduled visit, including the initial assessment and initial visit, the Nurse Consultant or Case Manager may request Area Administrator approval to complete the visit outside the home. This approval must be documented in the participant's narrative. In rare cases, the Case Manager's cell phone may be used as a last resort to check in and out of the visit if the mobile app is unavailable.

A unit of service is one (1).

Advanced Pest Control service is exclusively for the treatment of bed bug infestation.

Advanced Pest Control cannot be authorized for a participant residing in an apartment complex, boarding house, or Community Residential Care Facility (CRCF). However, authorization may be considered for a participant residing in a single dwelling rental house or a duplex dwelling rental house that is not managed by a governmental entity such as the Housing Authority.

In the case of a rental single house or rental duplex and the landlord does not provide Advanced Pest Control (bed bug infestation treatment) the **Case Manager** must obtain permission from the participant and/or Authorized Representative to speak to the landlord about Advanced Pest Control treatment.

If the participant and/or Authorized Representative refuses to allow the **Case Manager** to contact the landlord, the **Case Manager** will deny the Advanced Pest Control request. This information must be clearly documented in the narrative including the participant and/or Authorized Representative's permission to contact the landlord, the landlord's full name, the contact with the landlord and the landlord's response. A CLTC Notification Form will be sent stating the reason the service is denied. An Appeals Notice will not be included when the service is denied for this reason.

If the landlord refuses to provide the Advanced Pest Control for the participant, the **Case Manager** will proceed with the approval and authorization process.

If the landlord refuses to allow Advanced Pest Control treatment to be provided, Advanced Pest Control should not be authorized, and the **Case Manager** will send CLTC Notification Form stating the reason the service is denied. An Appeals Notice will not be included when the service is denied for this reason.

The need for Advanced Pest Control must be documented in the Home Assessment section in Phoenix.

The **Case Manager** will update the Service Plan to include the need for Advanced Pest Control.

The **Case Manager** requests initial approval in the Prior Approval section of the Service Plan. The CLTC Area Administrator/Lead Team (or MMP Care Coordinator for MMP participants) approves the initial request to allow the case manager to make referral to chosen provider.

The **Case Manager** explains to the participant and/or Authorized Representative that if a Pest Control provider is already in place and if that Pest Control provider is also an Advanced Pest Control provider, the same provider will provide the Advanced Pest Control (bedbug infestation treatment) if the treatment is approved.

If the participant does not currently receive Pest Control service or the current Pest Control provider does not also provide Advanced Pest Control, the **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in order of preference. The choices may be obtained verbally, by mail or during an in-home visit. If the choices are made verbally, the choices are documented in the narrative in order of preference. The **Case Manager** will create a referral to the chosen providers for Advanced Pest Control in order of preference in Phoenix.

NOTE: The Case Manager will not create the service authorization until the Area Office Designee has approved the request and the provider has confirmed bedbug eradication.

The Advanced Pest Control provider visits the participant's home to verify bed bug infestation. If bedbug infestation is confirmed, Advanced Pest Control provider **enters the following information into Phoenix:**

- Confirms bedbug infestation;
- Informs the specific type of treatment to be used;
- Provides photo or photos of bedbugs found;
- States the total cost of treatment;
- Describes the steps that must be taken by the participant in preparation for successful treatment;
- Describes the likelihood of successful eradication;
- Agrees to provide fitted mattress covers for affected areas;
- Provides a one-year written warranty of service.

When the Advanced Pest Control provider provides all of the above required documentation, the Area Office Administrator will verify the bid is less than \$1,000 and the warranty is included, then will approve or deny the request in Phoenix. :

- Case manager follows up with the provider and verifies the home is ready for treatment.
- Discusses the option of authorizing Personal Care services with the participant and/or Authorized Representative if help is needed to prepare the home for successful treatment.
- Send CLTC Notification Form to the participant, including the Appeals Notice, if the service is denied.
- **The service provision form for Advanced Pest Control will be completed in Phoenix when notification is received from the Advanced Pest Control Provider that bedbugs have been eradicated.** The start date is the date the notification of completed treatment and eradication and the end date is thirty (30) days from the start date. The dollar amount entered is the dollar amount that was approved by Area Office Administrator.

The cost of Advanced Pest Control is \$1,000 and is not included in the \$7,500 lifetime cap for Environmental Modification Service.

Advanced Pest Control may only be authorized once in a lifetime. If a re-infestation occurs and is impacting the health, safety and welfare of a participant, then a request for exception may be sent to the Waiver Exceptions Group at WES@scdhhs.gov for consideration. The Case Manager must document the Waiver Exception denial/approval in the narrative once received.

The Advanced Pest Control provider must return to the home and provide re-treatment as necessary within the one (1) year warranty timeframe which begins on the start date of the authorization.

When re-treatment is necessary, the Case Manager will complete the additional follow-up tab for documentation in Phoenix. The Advanced Pest Control provider will document subsequent treatment in Phoenix.

7I Home Delivered Meals

A unit of service is one (1) meal delivered to a participant's residence. Each meal provides a minimum of one-third the current recommended dietary allowance (RDA) for the age group as adopted by the United States Department of Agriculture.

A maximum of 14 meals may be authorized per week (2 meals/ day), according to the participant's needs. If the participant attends Adult Day Health Care, a meal should not be provided while the participant is at the facility. For example, if the participant attends daycare 3 days per week, he/she will only be authorized 11 meals per week. 14 maximum per week – 3 meals provided at daycare = 11 meals may be authorized.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in the order of preference. The choice may be obtained verbally, by mail or during an in-home visit. The choices will be documented in the narrative in order of preference.

The **Case Manager** will update the Service Plan to include the need for Home Delivered Meals.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Service Justification: Focus on nutritional needs and/or inability to afford nutritional foods and/or inability to physically prepare a meal, etc.

The **Case Manager** will create a referral to the chosen providers in order of preference via Phoenix.

No Physician's Order is required for this service.

When the referral is accepted, the **Case Manager** will create the Service Provision Form.

The **Case Manager** will authorize the amount, frequency, and duration of the services in accordance with the participant's Service Plan. The start date for this service will be Sunday.

The Service Provision Form must be sent to the provider via Phoenix to initiate services. This service does not require a Service Plan be sent to the provider.

7J Nutritional Supplements

A unit of service is one (1) case of Nutritional Supplements per month.

Waiver participants who have a need for nutritional supplements, may be authorized for up to two (2) cases per month. Two cases per month is the maximum amount allowed. No exceptions may be made.

The Nutritional Supplement must contain at least two hundred twenty (220) calories per two hundred fifty (250) milliliters.

The **Case Manager** will update the Service Plan to include the need for Nutritional Supplements.

Prior approval must be requested and approved via the service request section of the Service Plan in Phoenix.

NOTE: Prescriber's Order Form is to be obtained PRIOR to submitting prior approval.

Service Justification: State that Prescriber's Order Form was received and scanned in Phoenix.

A Physician's Order is required for this service. The SCDHHS Prescriber's Order Form must be completed by the participant's physician in order for this service to be authorized. Order must be signed by a primary authorized healthcare provider. Note: Physician Assistant and Nurse Practitioner signatures ARE acceptable.

The primary authorized healthcare provider must indicate the needs for the supplement and indicate at least one of the qualifying conditions:

1. Wasting (loss of ten percent (10%)) body mass in the last sixty (60) days.
2. Severe dental or gum problems that prevent the participant from chewing.
3. Has a condition that requires a protein supplement.
4. Has a swallowing problem that prevents the participant from achieving adequate weight.
5. Due to a medical condition, the participant cannot maintain adequate weight.

If it is determined that a participant does not meet the criteria for Nutritional Supplements but needs the service, a request for an exception may be sent to the Central Office Waiver Exceptions Group at WES@scdhhs.gov for review.

The Case Manager must document the Waiver Exception denial/approval in the narrative once received.

Note: If an exception needs to be approved by the **Central Office Waiver Exceptions Group**, the **Area Office** reviewer will review the request and enter a comment regarding the service justification in the comment section. The assigned **Case Manager** will send an email requesting the **Central Office Waiver Exceptions Group** to review the approval request. Once a response is received from the Waiver Exceptions Group, the Case Manager must document the exception approval/denial in a narrative.

When the **Central Office Waiver Exceptions Group** has granted an exception for Nutritional Supplements, an additional review is not required by **Central Office** upon re-evaluation if the following conditions are met:

- The original Central Office approval was not time limited.
- There is no change in the condition(s) which justified the need for the original exception approval.
- There has been no change in the participant's level of care.

If the participant is receiving tube feedings, Nutritional Supplements may not be authorized. Tube feeding supplies should be obtained through Medicaid State Health Plan. Any exceptions to this must be approved by Central Office.

The **Case Manager** will create the Service Provider Report (provider choice form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in the order of preference. The choice may be obtained verbally, by mail or during an in-home visit. The choices will be documented in the narrative in order of preference.

The **Case Manager** will create a referral to the chosen provider in order of preference in Phoenix.

The **Case Manager** will authorize the amount, frequency, and duration of the service in accordance with the participant's service plan. The completed Prescriber's Order Form must be scanned into Phoenix Scans prior to creating the service provision form for this service.

The Service Provision Form must be sent to the provider via Phoenix to initiate service. **A copy of the signed Prescriber's Order Form must be faxed or mailed to the provider.**

This service does not require a Service Plan be sent to the provider.

A new Prescriber's Order Form is not required at the re-evaluation. However, if the participant's condition changes and the change impacts the need for Nutritional Supplement (type), a new Prescriber's Order Form must be obtained.

If the participant's waiver application is terminated and then re-enrolled and the need for Nutritional Supplements continues, a new Prescriber's Order Form must be obtained. If the service for Nutritional Supplements is terminated for any reason, a new Prescriber's Order Form must be obtained to re-start the service.

There are two types of nutritional supplements that can be authorized:

1. Diabetic nutritional Supplements
2. Regular Nutritional Supplements case.

7K Personal Care Services

A unit of service is one (1) hour. This service can be authorized in one-half (1/2) hour in increments above a one (1) hour minimum.

Personal Care Services service is provided in the participant residence (except when shopping, laundry service, etc. must be done off-site or escort services are provided.)

Personal Care Services is not allowed to provide any skilled service at any time under any circumstances.

The **Case Manager** will update the Service Plan to include the need for Personal Care Services.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Service Justification: Include a condition or conditions affecting participant's ability to perform ADL's and/or IADL's. (Examples: Gait disturbance related to Parkinson's Disease; poor balance and fall risk due to vertigo; assistance with housekeeping and chores due to chronic pain from spinal stenosis; tires easily due to CHF; poor fine motor skills caused by rheumatoid arthritis). Include circumstances that may indicate need for high or increased service levels, such as recent hospitalization, physical decline of participant or caregiver, caregiver's need for assistance with care, or loss of caregiver. **Note: Do not include number of hours, desired time of day, or desired days of week.**

No Physician's Order is required for this service.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choices from the participant and/or Authorized Representative in the order of preference. The choices may be obtained verbally, by mail, or during an in-home visit. The choices will be documented in the narrative in order of preference.

The **Case Manager** will create a referral to the chosen provider in order of preference in Phoenix.

The **Case Manager** will authorize the amount, frequency, and duration of the service in accordance with the participant's Service Plan.

The authorization for this service is specific. The PCPC service authorization must be specific regarding the preferred time of day – morning, afternoon, or evening - that the service is need. These times are defined as:

Morning 3:00 am – 12:00 noon

Afternoon 12:01pm – 6:00 pm
Evening 6:01pm – 2:59 am

The time of day is negotiated with the provider and participant as necessary and the agreed upon time of day must be indicated on the Service Provision Form. The specific time of day for the service (such as 10:00am to 12:00pm) should appear on the Service Provision Form ONLY if it is essential in meeting the participant's service needs.

If the participant receives the service more than one (1) time per day, each appropriate time should be indicated on the Service Provision Form and a comment must be made in the comment section. The participant whose needs are such that the absence of the Personal Care Services would have a substantial impact on the participant's health and safety is identified as being "At Risk for Missed PC Visit." The **Case Manager** must discuss any special needs or preferences of the participant with the PC provider prior to authorization. In these cases, the Service Provision Form will state that the participant is "At Risk for Missed PC Visit."

It is acceptable to authorize Personal Care Services service for a participant on the date of admission to or discharge from an institutional such as a hospital, nursing home, rehabilitation center, or psychiatry facility. The record must clearly document the need for such service.

The Service Provision Form must be sent to the provider via Phoenix to initiate service. The initial Service Plan and the new Service Plan created following re-evaluation will be available to the provider via Phoenix. **If a revision is made to the Service Plan that affects the provider's service delivery, the provider must be notified of the change via phone or conversation feature.**

Service level approvals are time limited until the next re-evaluation or as indicated by the Reviewer in approval comments.

7L Personal Emergency Response System

A unit of service is one (1) month.

A Personal Emergency Response System (PERS) is an electronic device which enables a participant to secure help in the event of an emergency.

PERS may be authorized for those participants with an identified need who lives alone or who are alone for a substantial period of time. A substantial period of time is defined as being alone six (6) or more consecutive hours three (3) or more days per week. Any deviation from these criteria requires **Central Office Designee** approval.

The **Case Manager** will update the Service Plan to include the need for Personal Emergency Response System.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Service Justification: State that participant lives alone or is alone at least six or more consecutive hours on three or more days per week.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choices(s) from the participant and/or Authorized Representative in the order of preferences. The choices may be obtained verbally, by mail, or during an in-home visit. The choices will be documented in the narrative in order of preference.

The **Case Manager** will create a referral to the chosen provider for PERS installation and PERS on going in order of preference in Phoenix.

The **Case Manager** will authorize the amount, frequency, and duration of the service in accordance with the participant's Service Plan.

The PERS Installation Service Provision Form will be completed with a start date and an end date. The start date and the end date for PERS installation authorization must be in the same month. The **Case Manager** should negotiate with the provider to make sure the installation occurs within the same month. Depending on the time of the month when prior approval is given for the service, the provider may not be able to install the PERS during the current month. In this case, authorization would need to be completed for the next month. The Service Plan is not sent to the provider for this service.

The PERS on-going Service Provision Form start date will be the same day as the PERS installment start date and will not have an end date.

If it is determined that a participant does not meet the criteria for Personal Emergency Response System but needs the service, a request for an exception may be sent to the **Central Office Waiver Service Exceptions email group** at WES@scdhhs.gov for review.

Note: If an exception needs to be approved by **Central Office**, the **Area Office** reviewer will review the request and enter a comment regarding the service justification in the comment section. The assigned **Case Manager** will send an email to the Waiver Exceptions Group requesting review and approval of the exceptions request. The Case Manager must document the Waiver Exception denial/approval in the narrative once received.

When **Central Office** has granted an exception for Personal Emergency Response System, an additional review is not required by **Central Office** upon re-evaluation if the following conditions are met:

- The original **Central Office** approval was not time limited.
- There is no change in the condition(s) which justified the need for the original exception approval.
- There has been no change in the participant's level of care.

7M Pest Control

A unit of service is one (1) every other month.

Pest Control cannot be authorized for a participant residing in an apartment complex or a Community Residential Care Facility (CRCF). However, authorization may be considered for a participant residing in a single dwelling rental house or a duplex dwelling rental house that is not managed by a governmental entity such as the Housing Authority.

In the case of a rental single house, rental duplex, boarding house or room rental and the landlord does not provide Pest Control, the **Case Manager** must obtain permission from the participant and/or Authorized Representative to speak to the landlord about Pest Control treatment. If the participant and/or Authorized Representative refuses to allow the **Case Manager** to contact the landlord, the **Case Manager** will deny the Pest Control request. A CLTC Notification Form will be sent stating the reason the service is denied. An Appeals Notice will not be included when the service is denied for this reason. The participant's permission to contact the landlord and the discussion with the landlord must be clearly documented in the narrative. This documentation is to include the full name, the contact with the landlord and the landlord's response.

If the landlord refuses to provide the Pest Control for the participant, the **Case Manager** will proceed with the approval and authorization process.

If the landlord refuses to allow Pest Control to be provided, Pest Control should not be authorized, and the **Case Manager** will send CLTC Notification Form stating the reason the service is denied. An Appeals Notice will not be sent when the service is denied for this reason.

The need for Pest Control must be documented in the Home Assessment section in Phoenix.

Pest control service does not cover the removal of snakes, termites, or rodents.

The **Case Manager** will update the Service Plan to include the need for Pest Control.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Service Justification: Specify type of pest. Include home ownership status. If renting, state landlord was contacted and will not provide -- but will allow – treatment.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choices from the participant and/or Authorized Representative in the order of preference. The choices may be obtained verbally, by mail or during and in-home visit. The choices will be documented in the narrative in order of preference.

The **Case Manager** will create a referral to the chosen providers for Pest Control in order of

preference in Phoenix.

The **Case Manager** will authorize the amount, frequency, and duration of the service in accordance with the participant's service plan.

The Service Provision Form must be sent to the provider via Phoenix to initiate the service. This authorization will be completed with a start date and will not have an end date.

Exception: If a one (1) time treatment is authorized the **Case Manager** will indicate a start date and an end date.

A maximum of six (6) treatments can be authorized within a twelve (12) month period, no exceptions will be granted. Pest Control may be authorized with a frequency of every other month at a maximum. Pest Control treatment must be completed by the provider within fourteen (14) calendar days of the acceptance of the authorization. This service does not require a Service Plan be sent to the provider.

In the event more than one participant resides in the same residence, Pest Control must only be authorized for one of the participants.

The cost of Pest Control service is not included in the \$7500.00 lifetime cap for Environmental Modification services.

7N Respite - Community Residential Care Facility

A unit of service is a patient day. A patient day is defined as a twenty-four (24) hour period including the day of admission and excluding the day of discharge. A participant may use twenty-eight (28) days of respite-Community Residential Care Facility per fiscal year July 1st through June 30th.

Participants may receive respite services in Community Residential Care Facilities (CRCFs) that are enrolled with SCDHHS as respite providers.

Participants not eligible for respite in the CRCF:

1. Participants who are dependent on oxygen or mechanical ventilation.
2. Participants who require tube feedings.
3. Participants who are diagnosed with dementia and have a history of wandering, unless prior approved by Central Office.
4. Participants who are at skilled level of care, unless prior approved by Central Office.

The **Case Manager** will update the Service Plan to include the need for Community Residential Care Facility Respite.

Prior approval must be requested via the service request section of the Service Plan.

Service Justification: State reason needed (family's vacation, caregiver hospitalization, etc.). Include the number of days and type of respite already used during that fiscal year.

The **Case Manager** will contact the participant's choice of provider for availability on requested dates.

A Physician's Order is required for this service.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choice(s) from the participant and/or Authorized Representative in the order of preference. The choices may be obtained verbally, by mail, or during an in-home visit. If the choices are made verbally, the choices are documented in the narrative in the order of preference.

A 2-step PPD is required prior to authorization of the service. The first step of the PPD must be completed prior to admission and the second step may be completed after admission.

The **Case Manager** must obtain the completed Respite Form, which includes the participant's medical history, physical examination report and Physician's Order. The Respite Form must be completed no more than thirty (30) days prior to respite admission date.

Note: If it is not possible to obtain the Respite Form prior to admission, the provider may accept the medical information from the CLTC Assessment in lieu of the Respite Form. In such cases, the provider must obtain the Respite Form within forty-eight (48) hours of the respite admission.

The **Case Manager** will authorize the amount, frequency, and duration of the services in accordance with the participant's Service Plan.

The authorization for this service is day specific. The start date is the date of admission and the end date is the day before the discharge date. The comment section of the authorization should specify the number of days the participant will be in respite as well as the discharge date.

The Service Provision Form must be completed and sent to the provider along with the Service Plan and Respite Form via fax or mail.

The reimbursement will include all those items and supplies associated with patient care except prescribed medications and personal items. These items cannot be billed by the provider to Medicaid or the participant.

Examples of items to be supplied by the provider include Durable Medical Equipment, nonprescription medications, med pads, suctioning equipment and supplies, tube feeding equipment and feeding supplies, dressing change supplies, ostomy supplies, catheters, and tracheostomy supplies.

Participant and/or Authorized Representative will be responsible for supplying prescribed medications and personal care items such as soap, mouthwash, deodorant, shampoo, and clothing during the participant's respite stay.

Participants are allowed a total of twenty-eight (28) days of Community Residential Care Facility Respite during a fiscal year, but no more than fourteen (14) of those days can be respite in a nursing home or hospital during a fiscal year.

7O Respite Care - Institutional (Nursing Home or Hospital)

A unit of service is a patient day. A patient day is defined as a twenty-four (24) hour period including the day of admission and excluding the day of discharge. A participant may use fourteen (14) days of Respite-Institutional (Nursing Home or hospital) per fiscal year July 1st through June 30th.

Participants may receive respite services in nursing homes or hospitals that are enrolled with SCDHHS as respite providers.

The **Case Manager** will update the Service Plan to include the need for Respite Care-Institutional.

Prior approval must be requested via the service request section in the Service Plan in Phoenix.

Service Justification: State reason needed (family's vacation, caregiver hospitalization, etc.). Include the number of days and type of respite already used during that fiscal year.

A Physician's Order is required for this service.

The **Case Manager** will create the Service Provider Report (Provider Choice Form) in Phoenix. The **Case Manager** will obtain the provider choices from the participant and/or Authorized Representative in the order of preference. The choices may be obtained verbally, by mail, or during and in-home visit. If the choices are made verbally, the choices are documented in the narrative in the order of preference.

The **Case Manager** will contact the participant's choice of provider for availability on requested dates.

A 2-step PPD is required prior to authorization of the service. The first step of the PPD must be completed prior to admission and the second step may be completed after admission.

The **Case Manager** must obtain the completed Respite Form which includes the participant's medical history, physical examination report and Physician's Order. **The Respite Form must be completed no more than five (5) days prior to the Institutional (nursing home or hospital) Respite admission.**

Note: If it is not possible to obtain the Respite Form prior to admission, the provider may accept the medical information from the CLTC Assessment in lieu of the Respite Form. In such cases, the provider must obtain the Respite Form within forty-eight (48) hours of the respite admission.

The **Case Manager** must contact Area Office staff to request a PASRR be completed by the **Nurse Consultant** prior to admission for the Respite – Institutional service for a participant who will be receiving Respite service in a nursing home. Respite PASRR application will be created by Area Office staff and request should not be sent to Centralized Intake for the PASRR application to be created.

The **Case Manager** will authorize the amount, frequency, and duration of the service in accordance with the participant's Service Plan.

The authorization for this service is day specific. The start date is the date of admission, and the end date is the day before the discharge date. The comment section of the authorization should specify the number of days the participant will be in respite as well as the discharge date.

The Service Provision Form must be completed and sent to the provider along with the Service Plan, Respite Form and PASRR via fax or mail.

The reimbursement will include all those items and supplies associated with patient care except prescribed medications and personal items. These items cannot be billed by the provider to Medicaid or the participant.

Examples of items to be supplied by the provider include Durable Medical Equipment, nonprescription medications, med pads, suctioning equipment and supplies, tube feeding equipment and feeding supplies, dressing change supplies, ostomy supplies, catheters, and tracheostomy supplies.

Participant and/or Authorized Representative will be responsible for supplying prescribed medication and personal care items such as soap, mouthwash, deodorant, shampoo, and clothing during the participant's respite stay.

Participants are allowed a total of twenty-eight (28) days of respite during a fiscal year, but no more than fourteen (14) of these days can be Institutional Respite in a nursing home or hospital.

7P Telemonitoring

A unit of service is a participant day of telemonitoring.

Telemonitoring is a service to monitor body weight, blood pressure, blood glucose level, oxygen saturation, and basic heart rhythm information, for Community Choices participants who meet specific criteria.

Community Choices Waiver participants must meet the following criteria in order to be considered for the Telemonitoring service:

1. Have a primary diagnosis of Insulin Dependent Diabetes Mellitus, Hypertension, Chronic Obstructive Pulmonary Disease, and/or Congestive Heart Failure; and
2. Have a history of at least two hospitalizations and/or emergency room visits in the past twelve (12) months; and
3. Be capable of using the telemonitoring equipment and transmitting the necessary data or have an individual available that is capable of utilizing the telemonitoring equipment and transmitting the necessary data to the telemonitoring provider; and
4. Have a primary care physician that approves the use of the telemonitoring service and who is solely responsible for receiving and acting upon the information via the telemonitoring service.

The **Case Manager** must complete the Community Choices Pre-Screening for telemonitoring Form to assure the participant meets the criteria.

If the participant meets the criteria and chooses to receive the service, the **Case Manager** will update the Service Plan to include the need for Telemonitoring.

The **Case Manager** will update the Service Plan to include the need for telemonitoring.

Prior approval must be requested via the service request section of the Service Plan in Phoenix.

Service Justification: State that Pre-screen has been completed and scanned in Phoenix.

A Physician Order is required for this service.

The **Case Manager** will contact the participant's choice of provider for availability. The **Case Manager** will send a copy of the Community Choices Pre-Screening for telemonitoring Form to the chosen provider when the referral is accepted.

The telemonitoring provider must contact the participant's physician to obtain the orders to participate utilizing the approved SCDDHS Form. Upon receipt of the orders, the telemonitoring provider must notify the **Case Manager** and negotiate a start date. If notification by the provider has not been received within thirty (30) days of the referral, the **Case Manager** must call the telemonitoring provider to follow up on the status.

The **Case Manager** will authorize the amount, frequency, and duration of services in accordance with the participant's Service Plan.

The Service Provision Form must be sent to the provider by fax or mail. The initial Service Plan and the new Service Plan created following re-evaluation will be sent to the provider by fax or mail.

If upon Re-evaluation, the participant no longer meets the initial criteria due to no hospitalization or no Emergency Department visits, the Telemonitoring service may continue.

7Q Community Residential Care Facility Interactions with Home and Community Based Waiver Programs November 2022

Incontinence Supplies can be authorized for participants residing in a residential care facility.

Waiver services that **may** be routinely authorized by CLTC, if appropriate:

Community Choices	HIV/AIDS
Case Management	Case Management
Personal Care Services up to 1 hour/day, 7 days/week	Personal Care Services up to 1 hour/day, 7 days/week
Nutritional Supplements	Nutritional Supplements

Waiver services that **may not** be routinely authorized by CLTC, but may be given special consideration on a case by case basis and require Central Office approval:

Community Choices	HIV/AIDS
-----	Medicaid Nursing

Waiver services that **will not** be authorized by CLTC:

Community Choices	HIV/AIDS
Personal Care	Personal Care
Adult Day Health Care	Adult Day Health Care
Adult Day Health Care Nursing	Adult Day Health Care Nursing
Attendant Care	Attendant Care
Home Delivered Meals	Home Delivered Meals
Pest Control/Advanced Pest Control	Pest Control/Advanced Pest Control
PERS	-----
Environmental Modification/Enhanced Environmental Modification	Environmental Modification/Enhanced Environmental Modification
Companion	Companion
Telemonitoring	-----

7RHospice Benefit Interactions

Waiver services that **may** be routinely authorized by CLTC, if appropriate:

Community Choices	HIV	Vent
Case Management	Case Management	Case Management
Home Delivered Meals	Home Delivered Meals	Home Delivered Meals
Nutritional Supplements	Nutritional Supplements	Nutritional Supplements
PERS	--	PERS
Pest Control	Pest Control	Pest Control

Waiver services that **may not** be routinely authorized by CLTC, but may be given special consideration on a case by case basis and require Central Office approval:

Community Choices	HIV	Vent
Environmental Modification	Environmental Modification	Environmental Modification
Enhanced Environmental Modifications	Enhanced Environmental Modifications	Enhanced Environmental Modifications
Advanced Pest Control	Advanced Pest Control	Advanced Pest Control

Waiver services that **will not** be authorized by CLTC:

Community Choices	HIV	Vent
Adult Day Health Care	--	--
Adult Family Home	--	--
Attendant	Attendant	Attendant
Companion	Companion	-----
Incontinence Supplies	Incontinence Supplies	Incontinence Supplies
Personal Care Services	Personal Care Services	Personal Care Services
--	Private Duty Nursing	Private Duty Nursing
Respite	--	Respite
Specialized Medical Equipment	Specialized Medical Equipment	Specialized Medical Equipment
Telemonitoring	-----	-----
Self-Directed Services	Self-Directed Services	Self-Directed Services